The ORM Video Training Handbook

Narration scripts and demo directions for producing AI-generated training videos
Operative Resource Manager · UNTH Ituku-Ozalla

What this is. Thirty-eight ready-to-shoot video scripts, organised by staff role. Every script is written to be pasted directly into an AI video generator. Each one names the exact screen, the exact button and the exact words to speak.

Coverage. 38 staff roles · 165 screens · the complete perioperative journey, from booking a case to the patient leaving recovery.

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Part 0 — Production bible

Read this once. It makes all thirty-eight videos feel like one series instead of thirty-eight strangers.

How to use this handbook

Every video has the same four blocks. **The brief** says who it is for and what they will be able to do afterwards. **The scene table** is the shot list: what is on screen, what moves, and the exact words spoken over it. **The narration** is written to be spoken, not read — short sentences, no sub-clauses, no jargon a new porter would not recognise. **The copy-paste prompt** is a single condensed block to drop straight into an AI video generator.

Produce them in order within a part. Later videos assume the viewer has seen the earlier ones in that part, and every part assumes Part 1.

Cast, setting and visual language

ELEMENT	SPECIFICATION
Presenter	One recurring Nigerian presenter in theatre scrubs, mid-30s, warm and unhurried. The same person in every video, so the series has a face. To camera for the opening and closing only; narrating over the screen in between.
Voice	Nigerian English, measured, about 130 words per minute. Confident, never patronising. This audience are clinicians. They are not being taught medicine, only where the buttons are.
Setting	A Nigerian teaching-hospital theatre suite. Green and blue scrubs, tiled corridors, trolleys, a nurses' station with a laptop, staff using phones. Bright, clean, busy but not chaotic.
Device on screen	A **phone** whenever the task is done standing up: rosters, milestones, check-in, calling for a patient. A **laptop** for desk work: booking, reports, publishing rosters. The app is mobile-first and the videos must look mobile-first.
Screen capture	Real screens, not mock-ups. Zoom to 150% on the field being discussed. Highlight the tap target a beat *before* the finger arrives, so the eye is already there.
Patient names	Always fictional; use the ones in these scripts. Never film a real folder number.
Titles	Lower third, sans-serif, deep blue. Module name left, role right.
Music	Soft instrumental under the opening and closing only. Silent under instruction — the words are the content.

Rules that keep attention

> **THE FIRST EIGHT SECONDS**
>
> Never open with "In this video we will look at...". Open with **the problem the viewer has already had this week**: "The patient was ready. The theatre was ready. Nobody had told the porter." Then the promise: "By the end of this video that will never happen on your list again."

- **One idea per sixty seconds.** If two ideas are competing, cut it into two videos.

- **Show the consequence, not the feature.** Not "tap Save", but "tap Save, and the pack provider's phone buzzes in the store tonight, while there is still time to find the mesh."
- **Name real people doing real jobs.** Sister Ngozi. Dr Okeke. Emeka the porter. Abstractions do not hold attention; people do.
- **Say the cost of the shortcut.** Every module has one tempting shortcut. Name it, and name what it costs, once, plainly.
- **Close with one action** for the next working day — not a summary.
- **Stay under four minutes.** Longer is watched once and never finished.

Part 1 — Everyone, whatever your job

Four videos every member of staff watches, from the theatre chairman to the newest cleaner.

V1 · Why this app exists

All staff | 2 min 30 s | Watch first

THE BRIEF

Not a features tour. This is the video that decides whether anybody watches the other thirty-seven. Its only job is to make a busy clinician believe the app is on their side.

THEY WILL BE ABLE TO

- Say in one sentence what the ORM is for.
- Name the three things it removes: the phone calls, the waiting, the surprises.
- Know that feedback is read and acted on.

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:20 Hook	Corridor, early morning. A surgeon holds a phone to one ear and a folder in the other hand, visibly repeating himself. Cut to an empty theatre, lights on, nobody in it. Wall clock: 09:40.	*Nine-forty in the morning. The theatre is empty, the list started at nine, and this is the fourth phone call. Is the patient ready? Has the consent been signed? Did anybody tell the blood bank? Every one of those calls is a question the hospital already knows the answer to. It just had no way of telling you.*
2 · 0:20–0:45 Promise	Presenter to camera, quiet corner of the suite.	*That is the whole reason this application exists. Not to give you more forms. To stop you chasing answers the building already has. It is called the Operative Resource Manager, the ORM, and it is not really a booking app. It is the theatre's shared memory.*
3 · 0:45–1:25 The journey	Animated timeline sliding left to right: Booking, Pre-op visit, Packs and drugs, Send for patient, Reception, Holding, Theatre, Recovery, Ward. Each node lights with a small staff avatar above it.	*Follow one patient. A surgeon books her. That single act tells the pharmacy what drugs to prepare, tells the store what to pack, tells the anaesthetist she is on tomorrow's list, and tells the ward she needs a visit tonight. Nobody made a phone call. In the morning the theatre asks for her by name over the radio. The porter's phone tells him the ward. Recovery already knows she is coming. Every one of those people is looking at the same truth at the same moment.*
4 · 1:25–1:55 What it removes	Three icons appear and are struck through in turn: a ringing phone, an hourglass, an exclamation mark.	*So it takes three things out of your day. The phone calls, because the information arrives before you ask. The waiting, because everyone prepares at the same time instead of one after another. And the surprises: the missing consent found at eight in the morning, the pack nobody made up, the anaesthetist nobody rostered. The app cannot fix those. What it does is show them to you the night before, while there is still time.*
5 · 1:55–2:15 Honesty	Presenter, closer.	*It will not be perfect. Some of it will annoy you. When it does, tell us. There is a Feedback button on every screen and it reaches the people who build this, not a drawer. Several things you will use this week exist because somebody on your list complained about them.*
6 · 2:15–2:30 Close	Theatre doors opening, list running, staff moving with purpose. Title card:	*Watch the video for your own role next. It is under four minutes, and it is the four minutes that stop the nine-forty*

	ORM · UNTH Ituku-Ozalla.	*phone call. One thing to do tomorrow: open the app and find your name.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

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V1 · Why this app exists

SHOT 01
IMAGE:     Corridor, early morning. A surgeon holds a phone to one ear and a folder in the other hand, visibly repeating himself. Cut to an empty theatre, lights on, nobody in it. Wall clock: 09:40.
MOTION:    Slow dolly, travelling with the subject at walking pace
NARRATION: Nine-forty in the morning. The theatre is empty, the list started at nine, and this is the fourth phone call. Is the patient ready? Has the consent been signed? Did anybody tell the blood bank? Every one of those calls is a question the hospital already knows the answer to. It just had no way of telling you.

SHOT 02
IMAGE:     Presenter to camera, quiet corner of the suite.
MOTION:    Slow push in to a steady mid-shot
NARRATION: That is the whole reason this application exists. Not to give you more forms. To stop you chasing answers the building already has. It is called the Operative Resource Manager, the ORM, and it is not really a booking app. It is the theatre's shared memory.

SHOT 03
IMAGE:     Animated timeline sliding left to right: Booking, Pre-op visit, Packs and drugs, Send for patient, Reception, Holding, Theatre, Recovery, Ward. Each node lights with a small staff avatar above it.
MOTION:    Slow pan left to right, following each element as it appears
NARRATION: Follow one patient. A surgeon books her. That single act tells the pharmacy what drugs to prepare, tells the store what to pack, tells the anaesthetist she is on tomorrow's list, and tells the ward she needs a visit tonight. Nobody made a phone call. In the morning the theatre asks for her by name over the radio. The porter's phone tells him the ward. Recovery already knows she is coming. Every one of those people is looking at the same truth at the same moment.

SHOT 04
IMAGE:     Three icons appear and are struck through in turn: a ringing phone, an hourglass, an exclamation mark.
MOTION:    Slow pan left to right, following each element as it appears
NARRATION: So it takes three things out of your day. The phone calls, because the information arrives before you ask. The waiting, because everyone prepares at the same time instead of one after another. And the surprises: the missing consent found at eight in the morning, the pack nobody made up, the anaesthetist nobody rostered. The app cannot fix those. What it does is show them to you the night before, while there is still time.

SHOT 05
IMAGE:     Presenter, closer.
MOTION:    Slow push in to a steady mid-shot
NARRATION: It will not be perfect. Some of it will annoy you. When it does, tell us. There is a Feedback button on every screen and it reaches the people who build this, not a drawer. Several things you will use this week exist because somebody on your list complained about them.

SHOT 06
IMAGE:     Theatre doors opening, list running, staff moving with purpose. Title card: ORM · UNTH Ituku-Ozalla.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
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NARRATION: Watch the video for your own role next. It is under four minutes, and it is the four minutes that stop the nine-forty phone call. One thing to do tomorrow: open the app and find your name.

VIDEO 1 - WHY THIS APP EXISTS (2 min 30 s)
STYLE: Nigerian teaching hospital theatre suite, warm documentary tone, staff in green and blue scrubs. Recurring Nigerian presenter, mid-30s, in scrubs.
SCENES:
1 (0:00-0:20) Corridor at dawn. Surgeon on the phone, folder in hand, repeating himself. Cut to an empty lit theatre, clock at 09:40.
2 (0:20-0:45) Presenter to camera, quiet corner.
3 (0:45-1:25) Animated horizontal timeline: Booking, Pre-op visit, Packs and drugs, Send for patient, Reception, Holding, Theatre, Recovery, Ward. Nodes light in sequence with staff avatars.
4 (1:25-1:55) Three icons struck through: ringing phone, hourglass, exclamation mark.
5 (1:55-2:15) Presenter, closer framing.
6 (2:15-2:30) Theatre doors open, list running. Title card "ORM - UNTH Ituku-Ozalla".
NARRATION: paste the narration column of the scene table, in order.
VOICE: Nigerian English, warm, measured, about 130 wpm. Soft instrumental under scenes 1, 2 and 6 only; silence elsewhere.

V2 · Getting onto the theatre Wi-Fi

All staff | 3 min | Before anything else

THE BRIEF

Nobody uses the app if they cannot get onto the network. This video is the single most-watched thing in the series, because every new member of staff needs it on their first morning. It must be slow, literal, and show a real phone.

THEY WILL BE ABLE TO

- Join the theatre Wi-Fi and get through the sign-in page unaided.
- Understand that they sign in with their **own ORM username and password** — there is no shared Wi-Fi key.
- Stay signed in for a full 24-hour shift, and walk between theatres without signing in again.
- Know what to do when the sign-in page does not appear.

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:18 Hook	A registrar standing in a corridor, jabbing at her phone, frowning. She walks ten paces, tries again. Signal bars are full but nothing loads.	*Full signal. Nothing loading. She has been trying for four minutes, and the case she needs to book is starting in twenty. This is the most common problem in the whole hospital, and it takes ninety seconds to solve for good.*
2 · 0:18–0:50 Join	Phone screen, 200% zoom. Settings → Wi-Fi. The theatre network appears in the list. Finger taps it. The phone connects; a small "Sign in to network" notification slides down. Finger taps the notification.	*Open your Wi-Fi settings and pick the theatre network. There is no password on the network itself, so it will connect straight away. Now watch for this: a notification that says sign in to network. Tap it. On most phones it appears on its own within a few seconds. That notification is the door.*
3 · 0:50–1:25 The portal	The captive portal page fills the screen: hospital name, a username field, a password field, one blue Log In button. Finger types a username, then a password shown as dots. Taps Log In. A brief spinner, then a success page.	*This is the captive portal. It is not a separate account. Type the same username and password you use for the ORM itself — your own name, your own password. That is deliberate. There is no shared Wi-Fi key in this hospital, because a shared key is a key that ends up written on a wall, and then anybody in the car park is on the theatre network. Your login is yours. Tap Log In, and you are through.*
4 · 1:25–1:55 It lasts a shift	Clock graphic sweeping 24 hours. Phone in a pocket; the screen locks; hours pass; screen unlocks and the app opens instantly with no portal.	*Now the part that matters. Once you are in, you stay in for twenty-four hours. Your phone can lock, go in your pocket, run out of battery and come back — you do not sign in again. It used to be different. One of our anaesthetists signed in fourteen times in three days. That is fixed. One sign-in covers a full shift and the night after it.*
5 · 1:55–2:25 Roaming	Overhead animation of the suite: a router icon, then several access points along the corridor joined by a line. A staff avatar walks from Theatre 1 to Theatre 4 and to recovery; the connection indicator stays solid green throughout.	*The signal comes from one router, and the access points along the corridors extend it. To your phone it is all one network. So you can walk from Theatre One to recovery, from recovery to the store, and the app keeps working. Your device is remembered for the day, so moving between theatres never puts the sign-in page back in front of you.*

6 · 2:25–2:50 If it fails	Three numbered cards appear in turn, each with a small illustration.	*If the sign-in page never appears, three things fix almost every case. One: turn Wi-Fi off and on again, and wait five seconds. Two: open your browser and go to any plain address that is not secure — the portal cannot interrupt a secure page, which is why typing a search sometimes does nothing. Three: switch off mobile data while you sign in, because your phone will quietly prefer the mobile network and never ask the portal for the door. If none of those work, it is not you. Tell the theatre manager.*
7 · 2:50–3:00 Close	The registrar from scene 1, now walking and booking on her phone, unhurried.	*One thing to do tomorrow: sign in once, at the start of your shift, before you need it. Then forget about it for the rest of the day.*

PRODUCTION NOTE

Film the portal page on a **real phone**, in portrait, with a real thumb. Every previous attempt to train this with a laptop screenshot failed, because the notification behaves differently on a phone and the notification is the part people miss.

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

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V2 · Getting onto the theatre Wi-Fi

SHOT 01
IMAGE:     A registrar standing in a corridor, jabbing at her phone, frowning. She walks ten paces, tries again. Signal bars are full but nothing loads.
MOTION:    Slow dolly, travelling with the subject at walking pace
NARRATION: Full signal. Nothing loading. She has been trying for four minutes, and the case she needs to book is starting in twenty. This is the most common problem in the whole hospital, and it takes ninety seconds to solve for good.

SHOT 02
IMAGE:     Phone screen, 200% zoom. Settings &rarr; Wi-Fi. The theatre network appears in the list. Finger taps it. The phone connects; a small "Sign in to network" notification slides down. Finger taps the notification.
MOTION:    Hold steady on the screen, no camera move; highlight the tap target a beat before the finger arrives
NARRATION: Open your Wi-Fi settings and pick the theatre network. There is no password on the network itself, so it will connect straight away. Now watch for this: a notification that says sign in to network. Tap it. On most phones it appears on its own within a few seconds. That notification is the door.

SHOT 03
IMAGE:     The captive portal page fills the screen: hospital name, a username field, a password field, one blue Log In button. Finger types a username, then a password shown as dots. Taps Log In. A brief spinner, then a success page.
MOTION:    Hold steady on the screen, no camera move; highlight the tap target a beat before the finger arrives
NARRATION: This is the captive portal. It is not a separate account. Type the same username and password you use for the ORM itself — your own name, your own password. That is deliberate. There is no shared Wi-Fi key in this hospital, because a shared key is a key that ends up written on a wall, and then anybody in the car park is on the theatre network. Your login is yours. Tap Log In, and you are through.

SHOT 04
IMAGE:     Clock graphic sweeping 24 hours. Phone in a pocket; the screen locks; hours pass; screen unlocks and the app opens instantly with no portal.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Now the part that matters. Once you are in, you stay in for twenty-four hours. Your phone can lock, go in your pocket, run out of battery and come back — you do not sign in again. It used to be different. One of our anaesthetists signed in fourteen times in three days. That is fixed. One sign-in covers a full shift and the night after it.

SHOT 05
IMAGE:     Overhead animation of the suite: a router icon, then several access points along the corridor joined by a line. A staff avatar walks from Theatre 1 to Theatre 4 and to recovery; the connection indicator stays solid green throughout.
MOTION:    Slow dolly, travelling with the subject at walking pace
NARRATION: The signal comes from one router, and the access points along the corridors extend it. To your phone it is all one network. So you can walk from Theatre One to recovery, from recovery to the store, and the app keeps working. Your device is remembered for the day, so moving between theatres never puts the sign-in page back in front of you.

SHOT 06
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IMAGE: Three numbered cards appear in turn, each with a small illustration.
MOTION: Slow pan left to right, following each element as it appears
NARRATION: If the sign-in page never appears, three things fix almost every case. One: turn Wi-Fi off and on again, and wait five seconds. Two: open your browser and go to any plain address that is not secure — the portal cannot interrupt a secure page, which is why typing a search sometimes does nothing. Three: switch off mobile data while you sign in, because your phone will quietly prefer the mobile network and never ask the portal for the door. If none of those work, it is not you. Tell the theatre manager.

SHOT 07
IMAGE: The registrar from scene 1, now walking and booking on her phone, unhurried.
MOTION: Slow dolly, travelling with the subject at walking pace
NARRATION: One thing to do tomorrow: sign in once, at the start of your shift, before you need it. Then forget about it for the rest of the day.

VIDEO 2 - GETTING ONTO THE THEATRE WI-FI (3 min)
STYLE: Nigerian teaching hospital corridors. Real phone in portrait, real thumb, 200% screen zoom, tap targets highlighted a beat early.
SCENES:
1 (0:00-0:18) Registrar in a corridor jabbing at her phone, full signal, nothing loading. She walks ten paces and tries again.
2 (0:18-0:50) Phone Wi-Fi settings; theatre network selected; connects; "Sign in to network" notification slides down; thumb taps it.
3 (0:50-1:25) Captive portal page: hospital name, username, password, one blue Log In button. Types credentials, taps Log In, brief spinner, success page.
4 (1:25-1:55) 24-hour clock sweep. Phone locks, goes in a pocket, hours pass, unlocks, app opens instantly with no portal.
5 (1:55-2:25) Overhead animation of the theatre suite: router plus access points along corridors, joined by cable. Staff avatar walks Theatre 1 to recovery to store; signal indicator stays solid green.
6 (2:25-2:50) Three numbered troubleshooting cards appearing in turn.
7 (2:50-3:00) The registrar from scene 1, now booking on her phone while walking, unhurried.
NARRATION: paste the narration column of the scene table, in order.
VOICE: Nigerian English, patient and literal. This is a first-morning video; do not rush it.

V3 · Why the network is controlled

All staff | 2 min 30 s | Pairs with V2

THE BRIEF

Staff will discover that the theatre Wi-Fi is not an open internet cafe, and some will be annoyed. Better they hear the reason from us, framed around the patient, than invent a worse explanation. The tone is respectful and adult throughout — explaining, never scolding.

THEY WILL BE ABLE TO

- Explain why the network is metered and monitored, in terms of clinical priority.
- Understand that their own account is tied to their activity, and what that means.
- Know what the network is for, and what to use their own mobile data for.
- Know that shared handsets do not mean shared accounts.

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:20 Hook	Split screen. Left: an emergency booking screen spinning, trying to save. Right: a phone in a side room streaming a football match at full quality.	*On the left, an emergency case is trying to reach the theatre. On the right, a match is streaming in the changing room. They are sharing one connection. Only one of them can win, and the hospital has to decide which.*
2 · 0:20–0:55 The reason	Presenter to camera. Behind, a bandwidth gauge animates: a wide green band labelled Clinical, a narrow band labelled Everything else.	*So the theatre network is controlled. Not to police you, and not to save money. It is controlled because an emergency booking, a blood request and a theatre alert have to get through at the exact moment somebody needs them, and a connection that everybody is streaming on cannot promise that. Clinical traffic comes first. Everything else shares what is left.*
3 · 0:55–1:25 What it is for	Two columns build up. Left, ticked: the ORM, clinical email, hospital systems, looking something up for a patient. Right, crossed: video streaming, large downloads, file sharing, gaming.	*Here is the honest line. The theatre Wi-Fi is for work. The ORM, hospital systems, looking something up because a patient in front of you needs it — all of that, freely, and nobody will ever question it. Streaming, big downloads, sharing large files: use your own data for those. Not because they are sinful. Because they are the difference between an emergency alert arriving in one second and arriving in forty.*
4 · 1:25–1:55 Your account	A simple log view: name, device, time signed in, time signed out. Deliberately unremarkable and boring-looking.	*Because you sign in as yourself, the network knows which account is connected and when. That is worth being plain about. It is not there to read your messages and nobody is watching your browsing. It exists so that when the network is struggling, somebody can see what is consuming it and fix it, and so that a device that should not be on a clinical network can be removed. It is the same reason theatre doors have locks.*
5 · 1:55–2:20 Shared phones	Two nurses at a workstation, one handing a shared ward handset to the other. On screen, one signs out and the next signs in.	*One more thing, because it happens here every day. Handsets get shared. That is fine — the phone can be shared. The account cannot. If you pick up a ward phone somebody else signed in on, sign out and sign in as yourself. Otherwise*

		your pre-operative visit gets recorded under their name, and when somebody asks who cleared that patient, the app will give the wrong answer.
6 · 2:20–2:30 Close	Presenter, warm.	*Use it for the patient and use it as much as you like. That is what it is for. One thing to do tomorrow: check whose name is at the top of the shared handset before you type anything into it.*

TONE CHECK BEFORE YOU RENDER

Read scene 4 aloud. If it sounds like a warning, rewrite it. Staff who feel watched stop recording things, and a theatre system with missing records is more dangerous than a slow one.

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

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V3 · Why the network is controlled

SHOT 01
IMAGE:     Split screen. Left: an emergency booking screen spinning, trying to save. Right: a phone in a side room streaming a football match at full quality.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: On the left, an emergency case is trying to reach the theatre. On the right, a match is streaming in the changing room. They are sharing one connection. Only one of them can win, and the hospital has to decide which.

SHOT 02
IMAGE:     Presenter to camera. Behind, a bandwidth gauge animates: a wide green band labelled Clinical, a narrow band labelled Everything else.
MOTION:    Slow push in to a steady mid-shot
NARRATION: So the theatre network is controlled. Not to police you, and not to save money. It is controlled because an emergency booking, a blood request and a theatre alert have to get through at the exact moment somebody needs them, and a connection that everybody is streaming on cannot promise that. Clinical traffic comes first. Everything else shares what is left.

SHOT 03
IMAGE:     Two columns build up. Left, ticked: the ORM, clinical email, hospital systems, looking something up for a patient. Right, crossed: video streaming, large downloads, file sharing, gaming.
MOTION:    Slow pan left to right, following each element as it appears
NARRATION: Here is the honest line. The theatre Wi-Fi is for work. The ORM, hospital systems, looking something up because a patient in front of you needs it — all of that, freely, and nobody will ever question it. Streaming, big downloads, sharing large files: use your own data for those. Not because they are sinful. Because they are the difference between an emergency alert arriving in one second and arriving in forty.

SHOT 04
IMAGE:     A simple log view: name, device, time signed in, time signed out. Deliberately unremarkable and boring-looking.
MOTION:    Slow push in
NARRATION: Because you sign in as yourself, the network knows which account is connected and when. That is worth being plain about. It is not there to read your messages and nobody is watching your browsing. It exists so that when the network is struggling, somebody can see what is consuming it and fix it, and so that a device that should not be on a clinical network can be removed. It is the same reason theatre doors have locks.

SHOT 05
IMAGE:     Two nurses at a workstation, one handing a shared ward handset to the other. On screen, one signs out and the next signs in.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: One more thing, because it happens here every day. Handsets get shared. That is fine — the phone can be shared. The account cannot. If you pick up a ward phone somebody else signed in on, sign out and sign in as yourself. Otherwise your pre-operative visit gets recorded under their name, and when somebody asks who cleared that patient, the app will give the wrong answer.

SHOT 06
IMAGE:     Presenter, warm.
MOTION:    Slow pull back, settling as the line lands
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NARRATION: Use it for the patient and use it as much as you like. That is what it is for. One thing to do tomorrow: check whose name is at the top of the shared handset before you type anything into it.

VIDEO 3 - WHY THE NETWORK IS CONTROLLED (2 min 30 s)
STYLE: Nigerian teaching hospital. Respectful, adult, explanatory. Never scolding.
SCENES:
1 (0:00-0:20) Split screen: left, an emergency booking screen spinning while trying to save; right, a phone streaming football at full quality in a changing room.
2 (0:20-0:55) Presenter to camera; behind, a bandwidth gauge with a wide green "Clinical" band and a narrow "Everything else" band.
3 (0:55-1:25) Two columns building: ticked - ORM, clinical email, hospital systems, looking something up for a patient; crossed - streaming, large downloads, file sharing, gaming.
4 (1:25-1:55) A plain, boring log view: name, device, sign-in time, sign-out time.
5 (1:55-2:20) Two nurses at a workstation sharing a ward handset; on screen one account signs out and another signs in.
6 (2:20-2:30) Presenter, warm close.
NARRATION: paste the narration column of the scene table, in order.
VOICE: Nigerian English, calm and respectful. Scene 4 must sound like an explanation, not a warning.

V4 · What follows you everywhere

All staff | 3 min

THE BRIEF

Five behaviours exist on every screen in the app. Teaching them once here saves explaining them in thirty-four later videos.

THEY WILL BE ABLE TO

- Understand the theatre radio, and why it ducks the music instead of stopping it.
- Trust that a half-finished form survives switching apps.
- Keep working when the internet drops, and know what happens to what they typed.
- Find the Feedback button and the Role Guide.

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:20 Hook	A nurse half-way through a long pre-op form. Her phone rings; she takes the call, opens WhatsApp to check a folder number, comes back eight minutes later. The form is exactly as she left it.	*She left this form for eight minutes. A call, then WhatsApp, then back. Watch what did not happen. Nothing disappeared. That used to be the single most hated thing about this app, and it is now the first thing you should know about it.*
2 · 0:20–0:55 Drafts	Same form. A small grey line under the header reads "Saved 09:12". Phone is locked, reopened; a soft banner says the entry was restored.	*Every long form keeps a private draft on your own phone as you type. Switch apps, lock the screen, let the battery die — when you come back it offers your work back to you. Two rules protect you. It is kept for a few hours only, so yesterday's half-typed case never reappears by accident. And it is tied to the person who typed it — on a shared ward handset, your draft is never handed to the next person who signs in.*
3 · 0:55–1:35 The radio	Speaker icon in a corridor. Soft music playing, waveform visible. An announcement begins; the music waveform drops low but does not stop; the announcement finishes and the music rises again.	*The theatre radio speaks announcements out loud: send for the patient, the patient has arrived, an emergency has been booked. If music is playing, it does not stop — it drops low underneath and comes back afterwards. That is deliberate. Music cutting dead in a theatre is itself a distraction, and people look up at the wrong moment. Only one screen speaks, even when the app is open on three computers at the same desk, so you hear an announcement once and not three times.*
4 · 1:35–2:15 Offline	Phone with the Wi-Fi symbol crossed out. The user saves a milestone; a small "queued" marker appears. Connection returns; the marker clears with a tick.	*The signal in this hospital is not perfect and the app is built for that. If you record something while the connection is down, it is held on your phone and sent the moment you are back — you do not have to remember it or type it twice. Two things to know. It waits about thirty seconds before telling you it could not reach the server, so give it that time before pressing again. And if you do press twice, the app recognises the repeat and will not book the same case two times.*
5 · 2:15–2:45 Help	Sidebar scrolls to the bottom: Role Guide, Staff Training, Feedback.	*Three things live at the bottom of the menu and almost nobody finds them. Role Guide tells you what your own role*

		is expected to do, in plain words. Staff Training holds these videos. Feedback goes straight to the people who build this. Use it. It is the shortest route between something annoying you and something changing.
6 · 2:45–3:00 Close	Presenter.	*One thing to do tomorrow: start a form, deliberately switch to another app, and come back. Once you have seen it survive, you will stop rushing, and rushing is what causes the mistakes.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

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V4 · What follows you everywhere

SHOT 01
IMAGE:     A nurse half-way through a long pre-op form. Her phone rings; she takes the call, opens WhatsApp to check a folder number, comes back eight minutes later. The form is exactly as she left it.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: She left this form for eight minutes. A call, then WhatsApp, then back. Watch what did not happen. Nothing disappeared. That used to be the single most hated thing about this app, and it is now the first thing you should know about it.

SHOT 02
IMAGE:     Same form. A small grey line under the header reads "Saved 09:12". Phone is locked, reopened; a soft banner says the entry was restored.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Every long form keeps a private draft on your own phone as you type. Switch apps, lock the screen, let the battery die — when you come back it offers your work back to you. Two rules protect you. It is kept for a few hours only, so yesterday's half-typed case never reappears by accident. And it is tied to the person who typed it — on a shared ward handset, your draft is never handed to the next person who signs in.

SHOT 03
IMAGE:     Speaker icon in a corridor. Soft music playing, waveform visible. An announcement begins; the music waveform drops low but does not stop; the announcement finishes and the music rises again.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: The theatre radio speaks announcements out loud: send for the patient, the patient has arrived, an emergency has been booked. If music is playing, it does not stop — it drops low underneath and comes back afterwards. That is deliberate. Music cutting dead in a theatre is itself a distraction, and people look up at the wrong moment. Only one screen speaks, even when the app is open on three computers at the same desk, so you hear an announcement once and not three times.

SHOT 04
IMAGE:     Phone with the Wi-Fi symbol crossed out. The user saves a milestone; a small "queued" marker appears. Connection returns; the marker clears with a tick.
MOTION:    Slow push in
NARRATION: The signal in this hospital is not perfect and the app is built for that. If you record something while the connection is down, it is held on your phone and sent the moment you are back — you do not have to remember it or type it twice. Two things to know. It waits about thirty seconds before telling you it could not reach the server, so give it that time before pressing again. And if you do press twice, the app recognises the repeat and will not book the same case two times.

SHOT 05
IMAGE:     Sidebar scrolls to the bottom: Role Guide, Staff Training, Feedback.
MOTION:    Slow push in
NARRATION: Three things live at the bottom of the menu and almost nobody finds them. Role Guide tells you what your own role is expected to do, in plain words. Staff Training holds these videos. Feedback goes straight to the people who build this. Use it. It is the shortest route between something annoying you and something changing.

SHOT 06
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IMAGE: Presenter.
MOTION: Slow pull back, settling as the line lands
NARRATION: One thing to do tomorrow: start a form, deliberately switch to another app, and come back. Once you have seen it survive, you will stop rushing, and rushing is what causes the mistakes.

VIDEO 4 - WHAT FOLLOWS YOU EVERYWHERE (3 min)
SCENES: 1 (0:00-0:20) Nurse mid-form, takes a call, opens WhatsApp, returns after 8 minutes; form intact. 2 (0:20-0:55) Same form with a small "Saved 09:12" line; phone locked and reopened; soft "entry restored" banner. 3 (0:55-1:35) Corridor speaker; music waveform ducks low under an announcement, then rises. 4 (1:35-2:15) Phone with Wi-Fi crossed out; a milestone saved shows "queued"; connection returns and it clears with a tick. 5 (2:15-2:45) Sidebar scrolling to Role Guide, Staff Training, Feedback. 6 (2:45-3:00) Presenter close.
NARRATION: paste the narration column, in order.
VOICE: Nigerian English, reassuring.

Part 2 — Surgical team

Surgeons, consultant surgeons and house officers. The list begins here: almost everything the rest of the hospital does tomorrow is decided by what is typed into these screens today.

V5 · Booking an elective case

Surgeon | Consultant surgeon | House officer | 4 min

THE BRIEF

The most important screen in the application. Every downstream department is triggered by this one form, and the commonest cause of a cancelled case is something left blank here. Teach it as a chain of consequences, not a list of fields.

THEY WILL BE ABLE TO

- Book a complete elective case, including packs and drugs, in one sitting.
- Explain why start time and expected duration are compulsory.
- Give the patient their pack codes before they leave the clinic.
- Recognise the outstanding-items banner and act on it the same day.

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:20 Hook	A cancelled case. Patient sitting on a trolley in a corridor, gown on, being wheeled back. Caption: "Cancelled — mesh not available".	*This patient fasted from midnight. She was cancelled at ten past nine because the mesh was not in the pack. Nobody did anything wrong on the morning. It was decided three weeks earlier, on the booking form, in about four seconds.*
2 · 0:20–0:55 Patient	Laptop. Surgeries → Book Surgery. Search field; typing a folder number brings up an existing patient. Then a demo of registering a new patient: name, folder number, age, sex, ward.	*Start in Surgeries, then Book Surgery. Search by folder number first — most patients are already in the system, and finding them keeps their whole history attached to this case. If it is a new patient, register them here: name, folder number, age, sex and ward. Take care with the folder number. It is the one thing the pharmacy, the blood bank and the ward all use to find her.*
3 · 0:55–1:35 The operation	Unit dropdown, then procedure with type-to-search. Indication field. Magnitude set to Major. Anaesthesia set to General. Date picker, then start time 09:00 and duration 240 minutes, both highlighted.	*Now the operation. Choose your unit, then the procedure — type two or three letters and it finds it. Write the indication as you would in the notes. Then two fields that people try to skip, and cannot. Start time, and how long you expect it to take. Both are compulsory, and here is why: a case with no start time cannot be judged late, and a case with no duration means the next case on that list cannot be scheduled after it. Guess honestly rather than leaving it. An honest four hours is far more useful than a hopeful ninety minutes.*
4 · 1:35–2:20 Packs	The consumables list auto-fills from the chosen procedure. Surgeon adds two extra items. Drug and dressing list beside it. Special needs toggles: blood transfusion, diathermy, C-arm.	*This is where the mesh was lost. When you pick the procedure, the app fills in the usual pack for it, and adds the standard theatre pack on top. Read it. If this patient needs something extra, add it now — you are the only person in the building who knows that. The moment you save, this list becomes a job on the pack provider's phone and the drug list becomes a job for pharmacy, tonight, while the store is still open. Tick the special needs too. Blood transfusion here is what tells the blood bank to expect a request.*
5 · 2:20–2:55 Safety	Pre-operative section: haemoglobin, potassium, sodium, creatinine, HBsAg, HCV, HIV, blood pressure.	*The pre-operative results. Fill in whatever you have. What you do not have, leave — the case will still book. That was a deliberate change: cases were being blocked at booking*

	Some filled, some empty. Consent section with an upload control.	*because a resident did not yet hold a result somebody else was carrying. What is missing does not disappear. It comes back as an outstanding item on the case, visible to everybody, until whoever holds it enters it.*
6 · 2:55–3:30 Save	Save pressed. Confirmation panel showing the case, an amber banner reading "CONSENT + 6 PRE-OP ITEMS OUTSTANDING", and two codes labelled consumable pack code and pharmacy code.	*Save. Three things now exist. The case is on tomorrow's list and everybody who needs to see it can. The amber banner tells you exactly what is still missing — treat that as your job list for today, not somebody else's for the morning. And these two codes are for the patient: write them on her card. She hands them in at the store and at the pharmacy, and they find her pack without a single phone call.*
7 · 3:30–4:00 Close	The corridor from scene 1, but the trolley goes forward into theatre. Presenter.	*Booking is not paperwork. It is the instruction the whole hospital follows tomorrow. One thing to do on your next list: before you close the form, read the amber banner out loud. If it mentions consent, fix it today.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

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V5 · Booking an elective case

SHOT 01
IMAGE:     A cancelled case. Patient sitting on a trolley in a corridor, gown on, being
wheeled back. Caption: "Cancelled — mesh not available".
MOTION:    Slow dolly, travelling with the subject at walking pace
NARRATION: This patient fasted from midnight. She was cancelled at ten past nine because the
mesh was not in the pack. Nobody did anything wrong on the morning. It was decided three
weeks earlier, on the booking form, in about four seconds.

SHOT 02
IMAGE:     Laptop. Surgeries &rarr; Book Surgery. Search field; typing a folder number brings
up an existing patient. Then a demo of registering a new patient: name, folder number, age,
sex, ward.
MOTION:    Hold steady on the screen, no camera move; highlight the tap target a beat before
the finger arrives
NARRATION: Start in Surgeries, then Book Surgery. Search by folder number first — most
patients are already in the system, and finding them keeps their whole history attached to
this case. If it is a new patient, register them here: name, folder number, age, sex and
ward. Take care with the folder number. It is the one thing the pharmacy, the blood bank and
the ward all use to find her.

SHOT 03
IMAGE:     Unit dropdown, then procedure with type-to-search. Indication field. Magnitude set
to Major. Anaesthesia set to General. Date picker, then start time 09:00 and duration 240
minutes, both highlighted.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Now the operation. Choose your unit, then the procedure — type two or three
letters and it finds it. Write the indication as you would in the notes. Then two fields that
people try to skip, and cannot. Start time, and how long you expect it to take. Both are
compulsory, and here is why: a case with no start time cannot be judged late, and a case with
no duration means the next case on that list cannot be scheduled after it. Guess honestly
rather than leaving it. An honest four hours is far more useful than a hopeful ninety
minutes.

SHOT 04
IMAGE:     The consumables list auto-fills from the chosen procedure. Surgeon adds two extra
items. Drug and dressing list beside it. Special needs toggles: blood transfusion, diathermy,
C-arm.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: This is where the mesh was lost. When you pick the procedure, the app fills in the
usual pack for it, and adds the standard theatre pack on top. Read it. If this patient needs
something extra, add it now — you are the only person in the building who knows that. The
moment you save, this list becomes a job on the pack provider's phone and the drug list
becomes a job for pharmacy, tonight, while the store is still open. Tick the special needs
too. Blood transfusion here is what tells the blood bank to expect a request.

SHOT 05
IMAGE:     Pre-operative section: haemoglobin, potassium, sodium, creatinine, HBsAg, HCV,
HIV, blood pressure. Some filled, some empty. Consent section with an upload control.
MOTION:    Slow push in
NARRATION: The pre-operative results. Fill in whatever you have. What you do not have, leave
— the case will still book. That was a deliberate change: cases were being blocked at booking
```

because a resident did not yet hold a result somebody else was carrying. What is missing does not disappear. It comes back as an outstanding item on the case, visible to everybody, until whoever holds it enters it.

SHOT 06
IMAGE: Save pressed. Confirmation panel showing the case, an amber banner reading "CONSENT + 6 PRE-OP ITEMS OUTSTANDING", and two codes labelled consumable pack code and pharmacy code.
MOTION: Gentle handheld, almost still — observational, not restless
NARRATION: Save. Three things now exist. The case is on tomorrow's list and everybody who needs to see it can. The amber banner tells you exactly what is still missing — treat that as your job list for today, not somebody else's for the morning. And these two codes are for the patient: write them on her card. She hands them in at the store and at the pharmacy, and they find her pack without a single phone call.

SHOT 07
IMAGE: The corridor from scene 1, but the trolley goes forward into theatre. Presenter.
MOTION: Slow dolly, travelling with the subject at walking pace
NARRATION: Booking is not paperwork. It is the instruction the whole hospital follows tomorrow. One thing to do on your next list: before you close the form, read the amber banner out loud. If it mentions consent, fix it today.

VIDEO 5 - BOOKING AN ELECTIVE CASE (4 min)
STYLE: laptop screen capture at 150%, tap targets highlighted a beat early; intercut with a Nigerian theatre corridor.
SCENES: 1 (0:00-0:20) A gowned patient wheeled back from theatre, caption "Cancelled - mesh not available". 2 (0:20-0:55) Surgeries > Book Surgery; search by folder number; registering a new patient. 3 (0:55-1:35) Unit, procedure type-to-search, indication, magnitude Major, anaesthesia General, date, start time 09:00, duration 240 - the last two highlighted. 4 (1:35-2:20) Consumables auto-filling from the procedure; two extra items added; drug list; special-needs toggles. 5 (2:20-2:55) Pre-op labs partly filled; consent upload control. 6 (2:55-3:30) Save; confirmation with amber banner "CONSENT + 6 PRE-OP ITEMS OUTSTANDING" and two patient codes. 7 (3:30-4:00) Trolley now going forward into theatre; presenter close.
NARRATION: paste the narration column, in order.

V6 · Emergency booking

Surgeon | House officer | Anaesthetist | 3 min 30 s

THE BRIEF

Booked under pressure, often one-handed, often at night. The form is a stepped wizard with a progress bar for exactly that reason. Emphasise that it can be finished in stages and that the theatre hears about it immediately.

THEY WILL BE ABLE TO

- Raise an emergency case in under two minutes and know the theatre has heard.
- Work through all the steps without the form closing early.
- Upload a signed consent rather than re-typing one.
- Understand why an emergency cannot be dated in the past.

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:20 Hook	Accident and emergency at night. A registrar with a phone in one hand, gloved. Urgency without chaos.	*Ten at night. Perforated bowel. He has one free hand and about ninety seconds before he is needed again. This form is built for exactly this moment, and it is built so that ninety seconds is enough.*
2 · 0:20–0:55 Step 1	Phone. Emergency Booking. A progress bar across the top showing five steps, step 1 filled. Patient details, unit, procedure, indication, priority, required-by time.	*Emergency Booking, first item in the menu. Notice the bar across the top: five steps, and you can see where you are at all times. Step one is the patient and the operation. Priority, and the time the surgery is needed by. That time cannot be in the past — the app will refuse it. It sounds obvious, but a mistyped date once put a case a week behind us, and the theatre heard the alert for a patient no screen could show.*
3 · 0:55–1:35 Sent	Save and send. On screen, a confirmation. Cut to the theatre: a speaker, and the emergency board lighting up with the patient. Cut to phones buzzing in three pockets.	*Save and send. Right now, before you have typed anything else, the theatre knows. It is announced on the radio, it appears on the emergency board, and the team's phones alert — loudly, even if the phone is locked and on silent, because that is what an emergency alert is for. Now — and this is the part people used to get wrong — the form does not close. Keep going.*
4 · 1:35–2:20 Packs then consent	Step: consumables and drugs. Then the next step: a single upload control for a signed consent form, with a camera option. A photograph of a signed paper consent is taken and attached.	*Packs come next, and deliberately before consent. The store needs the longest warning, so it is asked for first. Then consent, and this step is only one thing: upload the signed form. Photograph the paper. Do not re-type it, do not summarise it — the signed sheet is the record, and a photograph of it is the fastest honest way to get it into the system at ten at night.*
5 · 2:20–2:55 Team	Team step: surgeon, assistant, anaesthetist. Then the completion screen with the case summary.	*Last, the team. Add who you know; leave what you do not. The case is already live, so this is refinement, not permission. And if you are called away half-way through, what you typed is kept — come back and finish it. An emergency booked in three visits to the form is still infinitely better than one held in your head.*

6 · 2:55–3:30 Close	The emergency board on a wall screen, the case sitting on it with its priority colour. Presenter.	*The moment you press save on step one, you have stopped being the only person who knows. One thing to remember: finish the remaining steps the same night, even from the corridor. The store cannot pack what it has not been told about.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

```
V6 · Emergency booking

SHOT 01
IMAGE:     Accident and emergency at night. A registrar with a phone in one hand, gloved. Urgency without chaos.
MOTION:    Slow push in
NARRATION: Ten at night. Perforated bowel. He has one free hand and about ninety seconds before he is needed again. This form is built for exactly this moment, and it is built so that ninety seconds is enough.

SHOT 02
IMAGE:     Phone. Emergency Booking. A progress bar across the top showing five steps, step 1 filled. Patient details, unit, procedure, indication, priority, required-by time.
MOTION:    Slow push in
NARRATION: Emergency Booking, first item in the menu. Notice the bar across the top: five steps, and you can see where you are at all times. Step one is the patient and the operation. Priority, and the time the surgery is needed by. That time cannot be in the past — the app will refuse it. It sounds obvious, but a mistyped date once put a case a week behind us, and the theatre heard the alert for a patient no screen could show.

SHOT 03
IMAGE:     Save and send. On screen, a confirmation. Cut to the theatre: a speaker, and the emergency board lighting up with the patient. Cut to phones buzzing in three pockets.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Save and send. Right now, before you have typed anything else, the theatre knows. It is announced on the radio, it appears on the emergency board, and the team's phones alert — loudly, even if the phone is locked and on silent, because that is what an emergency alert is for. Now — and this is the part people used to get wrong — the form does not close. Keep going.

SHOT 04
IMAGE:     Step: consumables and drugs. Then the next step: a single upload control for a signed consent form, with a camera option. A photograph of a signed paper consent is taken and attached.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Packs come next, and deliberately before consent. The store needs the longest warning, so it is asked for first. Then consent, and this step is only one thing: upload the signed form. Photograph the paper. Do not re-type it, do not summarise it — the signed sheet is the record, and a photograph of it is the fastest honest way to get it into the system at ten at night.

SHOT 05
IMAGE:     Team step: surgeon, assistant, anaesthetist. Then the completion screen with the case summary.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Last, the team. Add who you know; leave what you do not. The case is already live, so this is refinement, not permission. And if you are called away half-way through, what you typed is kept — come back and finish it. An emergency booked in three visits to the form is still infinitely better than one held in your head.

SHOT 06
IMAGE:     The emergency board on a wall screen, the case sitting on it with its priority colour. Presenter.
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MOTION: Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: The moment you press save on step one, you have stopped being the only person who knows. One thing to remember: finish the remaining steps the same night, even from the corridor. The store cannot pack what it has not been told about.

VIDEO 6 - EMERGENCY BOOKING (3 min 30 s)
STYLE: night, A&E, phone held one-handed. Urgent but controlled.
SCENES: 1 (0:00-0:20) Registrar at night, gloved, phone in one hand. 2 (0:20-0:55) Phone: Emergency Booking, five-step progress bar, step 1 - patient, unit, procedure, indication, priority, required-by time. 3 (0:55-1:35) Save and send; cut to theatre speaker, emergency board lighting up, three phones buzzing. 4 (1:35-2:20) Packs and drugs step; then consent step - a single upload control; photographing a signed paper consent. 5 (2:20-2:55) Team step; completion summary. 6 (2:55-3:30) Wall-mounted emergency board showing the case; presenter close.
NARRATION: paste the narration column, in order.

V7 · Consent and pre-op blocks

Surgeon | House officer | Ward nurse | 3 min

THE BRIEF

Consent is the single commonest reason a case is held in the morning. Two different people can settle it, and most staff know only one of the routes.

THEY WILL BE ABLE TO

- Record consent as a surgeon, and clear the morning block.
- Record consent as a nurse during the pre-operative visit, and know that this also settles it.
- Read the outstanding-items banner and tell which items block calling the patient.

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:18 Hook	08:05. A theatre nurse at a screen, phone to ear, patient not sent for. Case card shows a red "Consent outstanding".	*Five past eight. The theatre is ready and the patient has not been sent for, because the app says the consent is outstanding. It was signed last night. Nobody told the app.*
2 · 0:18–1:00 Surgeon route	Case → Consent. Upload the signed form, or record an electronic signature. Save. The red banner turns green.	*If you are the surgeon, open the case and go to Consent. Two ways in: upload a photograph of the signed paper, or record it signed electronically. Either satisfies it. Save, and the block clears immediately — the morning team can call for the patient without ringing anybody. Consultants and registrars both have this. If you were told you did not have permission, that was a fault, and it is fixed.*
3 · 1:00–1:45 Nurse route	Pre-Operative Visit form on a phone at the bedside. A checkbox: consent present and signed. Ticked. Save.	*And if you are the nurse doing the pre-operative visit the night before, you can settle it too. When you confirm on the visit form that the consent is present and signed, the app takes that as settled. You are standing at the bedside holding the folder — you are a better witness to that than anybody at a desk. This is the route most people do not know exists, and it is the one that prevents the eight o'clock phone call.*
4 · 1:45–2:25 Refusal	The same tick box set to "refused". The case shows a firm red state that does not clear.	*One exception, and it is absolute. If consent has been refused, nothing overrides it. Not a later upload, not a tick, not a manager. A refusal stands until the patient themselves changes it. The app is built that way on purpose, and you should expect it to argue with you.*
5 · 2:25–2:50 The other items	Banner expanded showing the outstanding list: haemoglobin, HIV, HBsAg, blood pressure, packs, drugs. Some marked as blocking, others informational.	*Consent is one of several outstanding items. The others — the bloods, the pack list, the drugs — do not stop the patient being called, but every one of them is somebody's job today. Open the banner. Whatever is on it is a phone call that will otherwise happen tomorrow morning at the worst possible time.*
6 · 2:50–3:00 Close	The nurse from scene 1, now sending for the patient calmly.	*One thing to do tonight: on your pre-operative round, tick the consent box while the folder is in your hand.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

```
V7 · Consent and pre-op blocks

SHOT 01
IMAGE:     08:05. A theatre nurse at a screen, phone to ear, patient not sent for. Case card shows a red "Consent outstanding".
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Five past eight. The theatre is ready and the patient has not been sent for, because the app says the consent is outstanding. It was signed last night. Nobody told the app.

SHOT 02
IMAGE:     Case &rarr; Consent. Upload the signed form, or record an electronic signature. Save. The red banner turns green.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: If you are the surgeon, open the case and go to Consent. Two ways in: upload a photograph of the signed paper, or record it signed electronically. Either satisfies it. Save, and the block clears immediately — the morning team can call for the patient without ringing anybody. Consultants and registrars both have this. If you were told you did not have permission, that was a fault, and it is fixed.

SHOT 03
IMAGE:     Pre-Operative Visit form on a phone at the bedside. A checkbox: consent present and signed. Ticked. Save.
MOTION:    Hold steady on the screen, no camera move; highlight the tap target a beat before the finger arrives
NARRATION: And if you are the nurse doing the pre-operative visit the night before, you can settle it too. When you confirm on the visit form that the consent is present and signed, the app takes that as settled. You are standing at the bedside holding the folder — you are a better witness to that than anybody at a desk. This is the route most people do not know exists, and it is the one that prevents the eight o'clock phone call.

SHOT 04
IMAGE:     The same tick box set to "refused". The case shows a firm red state that does not clear.
MOTION:    Slow push in
NARRATION: One exception, and it is absolute. If consent has been refused, nothing overrides it. Not a later upload, not a tick, not a manager. A refusal stands until the patient themselves changes it. The app is built that way on purpose, and you should expect it to argue with you.

SHOT 05
IMAGE:     Banner expanded showing the outstanding list: haemoglobin, HIV, HBsAg, blood pressure, packs, drugs. Some marked as blocking, others informational.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Consent is one of several outstanding items. The others — the bloods, the pack list, the drugs — do not stop the patient being called, but every one of them is somebody's job today. Open the banner. Whatever is on it is a phone call that will otherwise happen tomorrow morning at the worst possible time.

SHOT 06
IMAGE:     The nurse from scene 1, now sending for the patient calmly.
MOTION:    Slow pull back to close
```

NARRATION: One thing to do tonight: on your pre-operative round, tick the consent box while the folder is in your hand.

VIDEO 7 - CONSENT AND PRE-OP BLOCKS (3 min)
SCENES: 1 (0:00-0:18) 08:05, theatre nurse on the phone, case card showing red "Consent outstanding". 2 (0:18-1:00) Case > Consent; upload signed form or record electronic signature; banner turns green. 3 (1:00-1:45) Pre-Operative Visit form on a phone at the bedside; "consent present and signed" ticked; saved. 4 (1:45-2:25) Same control set to "refused"; a firm red state that will not clear. 5 (2:25-2:50) Outstanding-items banner expanded: haemoglobin, HIV, HBsAg, blood pressure, packs, drugs. 6 (2:50-3:00) The nurse from scene 1 calmly sending for the patient.
NARRATION: paste the narration column, in order.

V8 · The house officer's day

House officer | 3 min

THE BRIEF

House officers do more data entry in this app than anyone else and receive the least training. Frame the whole video around their own board, so the app becomes a to-do list rather than a filing duty.

THEY WILL BE ABLE TO

- Open their own board and see only their cases.
- Clear outstanding pre-op items in one pass.
- Chase the right person by WhatsApp from inside the app.

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:20 Hook	A house officer with three folders under one arm, phone in the other hand, walking fast.	*Six patients, four wards, one ward round, and every consultant assumes you are holding all of it in your head. You do not have to. The app already knows which cases are yours.*
2 · 0:20–1:00 My board	Phone. Dashboard opens on a personal board: only this user's cases, each with its outstanding items listed underneath.	*Your dashboard shows the cases you are linked to — the ones you booked and the ones where you are on the team. Every case carries its own list of what is missing. That is your round, written for you, in the order it matters. Work down it, not across the ward.*
3 · 1:00–1:50 Clearing items	A case opened. Haemoglobin typed in, sample time recorded. HIV and HBsAg set to negative. Each item disappears from the list as it is entered. The banner count drops from six to two.	*Tap any case and fill in what you now have. Haemoglobin and the time the sample was taken. The viral screen. The blood pressure. Each one you enter drops off the list in front of you, and the count goes down. You will feel it. That count is also what your consultant sees, so a case sitting at six outstanding items on the evening before is a conversation you would rather not have.*
4 · 1:50–2:30 Chasing	A case where the booking was made by someone else. Card shows the booker's name, role and phone, with a "Chase on WhatsApp" button. Tapped, it opens WhatsApp with a message already written.	*Some things are not yours to fix. When a case has outstanding items and you are not the person who can settle them, the card tells you who booked it, their role and their number — and Chase on WhatsApp writes the message for you, with the patient, the operation and exactly what is missing. Fifteen seconds instead of a ten-minute hunt for a phone number.*
5 · 2:30–3:00 Close	End of day, the board showing all items cleared. Presenter.	*Every item you clear today is a case that will not be delayed tomorrow, and everyone will assume the morning simply went well. One thing to do tomorrow: open your board before the ward round, not after it.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

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V8 · The house officer's day

SHOT 01
IMAGE:     A house officer with three folders under one arm, phone in the other hand, walking fast.
MOTION:    Slow dolly, travelling with the subject at walking pace
NARRATION: Six patients, four wards, one ward round, and every consultant assumes you are holding all of it in your head. You do not have to. The app already knows which cases are yours.

SHOT 02
IMAGE:     Phone. Dashboard opens on a personal board: only this user's cases, each with its outstanding items listed underneath.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Your dashboard shows the cases you are linked to — the ones you booked and the ones where you are on the team. Every case carries its own list of what is missing. That is your round, written for you, in the order it matters. Work down it, not across the ward.

SHOT 03
IMAGE:     A case opened. Haemoglobin typed in, sample time recorded. HIV and HBsAg set to negative. Each item disappears from the list as it is entered. The banner count drops from six to two.
MOTION:    Hold steady on the screen, no camera move; highlight the tap target a beat before the finger arrives
NARRATION: Tap any case and fill in what you now have. Haemoglobin and the time the sample was taken. The viral screen. The blood pressure. Each one you enter drops off the list in front of you, and the count goes down. You will feel it. That count is also what your consultant sees, so a case sitting at six outstanding items on the evening before is a conversation you would rather not have.

SHOT 04
IMAGE:     A case where the booking was made by someone else. Card shows the booker's name, role and phone, with a "Chase on WhatsApp" button. Tapped, it opens WhatsApp with a message already written.
MOTION:    Hold steady on the screen, no camera move; highlight the tap target a beat before the finger arrives
NARRATION: Some things are not yours to fix. When a case has outstanding items and you are not the person who can settle them, the card tells you who booked it, their role and their number — and Chase on WhatsApp writes the message for you, with the patient, the operation and exactly what is missing. Fifteen seconds instead of a ten-minute hunt for a phone number.

SHOT 05
IMAGE:     End of day, the board showing all items cleared. Presenter.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Every item you clear today is a case that will not be delayed tomorrow, and everyone will assume the morning simply went well. One thing to do tomorrow: open your board before the ward round, not after it.
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VIDEO 8 - THE HOUSE OFFICER'S DAY (3 min)
SCENES: 1 (0:00-0:20) House officer with three folders and a phone, walking fast. 2 (0:20-1:00) Phone dashboard showing a personal board of only their cases, each with outstanding items
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beneath. 3 (1:00-1:50) A case opened; haemoglobin, sample time, HIV, HBsAg entered; items vanish;
banner count drops 6 to 2. 4 (1:50-2:30) A case booked by someone else showing booker name, role,
phone, and a "Chase on WhatsApp" button that opens a pre-written message. 5 (2:30-3:00) End of
day, board clear; presenter close.
NARRATION: paste the narration column, in order.
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V9 · The consultant's oversight

Consultant surgeon | 3 min

THE BRIEF

Consultants will not do data entry, and should not be asked to. Sell this entirely as oversight: seeing the firm's whole workload without asking anyone.

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:18 Hook	A consultant in an office, being handed a paper list by a registrar. The list is handwritten, amended twice.	*This list was accurate when it was written. That was yesterday afternoon. Two cases have been added since, one has been cancelled, and nobody has told him. There is a version of it that is never out of date.*
2 · 0:18–1:00 Supervision	Surgeries list filtered by unit; cases show "Consultant: [name]" on each card.	*Every case names its supervising consultant, so filtering by your unit gives you your firm's real workload — today, tomorrow, next week. Cases your registrars booked appear the moment they book them, not when somebody remembers to tell you.*
3 · 1:00–1:45 My Practice	My Practice screen: case counts, case mix, on-time starts, cancellations with reasons.	*My Practice is your own record: how many cases, what mix, how many started on time, how many were cancelled and why. This is not surveillance — nobody else is browsing it. It is the number you need when you are asked to justify a theatre slot, and it is built from timestamps rather than memory.*
4 · 1:45–2:30 Governance	QA Review screen; then the Mortality Registry with an entry being opened.	*Two more, for the parts of the job nobody enjoys. QA Review holds cases flagged for discussion, with the full timeline attached — who was present, when the knife went to skin, what was delayed. And the Mortality Registry is the formal record. Both exist so that a morbidity meeting argues about the case rather than about what happened.*
5 · 2:30–3:00 Close	Consultant closing the laptop, walking into theatre. Presenter.	*You will use four screens and never fill in a form. One thing to do this week: open My Practice and look at your cancellation reasons. Two of them will be fixable by your own team, on the booking form.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

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V9 · The consultant's oversight

SHOT 01
IMAGE:     A consultant in an office, being handed a paper list by a registrar. The list is handwritten, amended twice.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: This list was accurate when it was written. That was yesterday afternoon. Two cases have been added since, one has been cancelled, and nobody has told him. There is a version of it that is never out of date.

SHOT 02
IMAGE:     Surgeries list filtered by unit; cases show "Consultant: [name]" on each card.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Every case names its supervising consultant, so filtering by your unit gives you your firm's real workload — today, tomorrow, next week. Cases your registrars booked appear the moment they book them, not when somebody remembers to tell you.

SHOT 03
IMAGE:     My Practice screen: case counts, case mix, on-time starts, cancellations with reasons.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: My Practice is your own record: how many cases, what mix, how many started on time, how many were cancelled and why. This is not surveillance — nobody else is browsing it. It is the number you need when you are asked to justify a theatre slot, and it is built from timestamps rather than memory.

SHOT 04
IMAGE:     QA Review screen; then the Mortality Registry with an entry being opened.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Two more, for the parts of the job nobody enjoys. QA Review holds cases flagged for discussion, with the full timeline attached — who was present, when the knife went to skin, what was delayed. And the Mortality Registry is the formal record. Both exist so that a morbidity meeting argues about the case rather than about what happened.

SHOT 05
IMAGE:     Consultant closing the laptop, walking into theatre. Presenter.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: You will use four screens and never fill in a form. One thing to do this week: open My Practice and look at your cancellation reasons. Two of them will be fixable by your own team, on the booking form.
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VIDEO 9 - THE CONSULTANT'S OVERSIGHT (3 min)
SCENES: 1 (0:00-0:18) Consultant handed a twice-amended handwritten list. 2 (0:18-1:00) Surgeries filtered by unit; each card naming its supervising consultant. 3 (1:00-1:45) My Practice: case counts, case mix, on-time starts, cancellations with reasons. 4 (1:45-2:30) QA Review with a case timeline; then the Mortality Registry. 5 (2:30-3:00) Consultant closing the laptop and walking into theatre.
NARRATION: paste the narration column, in order.
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Part 3 — Anaesthesia

Anaesthetists, consultant anaesthetists and anaesthetic technicians. This part carries one rule that must be said out loud in every video: a planned case is covered by the anaesthetist rostered to its specialty, or by nobody.

V10 · The anaesthetist board

Anaesthetist | Consultant anaesthetist | 3 min 30 s

THEY WILL BE ABLE TO

- Open the board and see every case they are covering tomorrow.
- Complete a pre-operative anaesthetic review from the ward.
- Spot an unfit patient the evening before rather than at the theatre door.

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:20 Hook	08:10, theatre door. An anaesthetist meets a patient for the first time; the observation chart shows a potassium of 6.9. The case is stood down.	*He is meeting this patient for the first time at the theatre door. Her potassium is six point nine. The case is now cancelled, the list has lost two hours, and every one of those facts was available last night.*
2 · 0:20–1:05 The board	Anaesthetist Board on a phone: tomorrow's cases grouped by theatre, each showing patient, procedure, anaesthesia type, and a coloured review status.	*The Anaesthetist Board is your list, not the surgeon's. Tomorrow's cases, grouped by theatre, each showing the operation, the planned anaesthetic and whether it has been reviewed. Open it the evening before. Twenty minutes with this screen is the difference between a list that runs and a morning spent apologising.*
3 · 1:05–2:00 The review	A case opened. Pre-op review form: airway assessment, ASA grade, comorbidities, current medication, the lab results already entered by the team, and a fitness decision with a free-text note.	*Tap a case for the pre-operative review. The bloods the surgical team entered are already here — you are not chasing them. Add the airway assessment, the ASA grade, the comorbidities and the drugs she is on. Then the decision: fit, fit with conditions, or not fit. Write the reason. That note is read by the surgeon, the theatre coordinator and whoever is on tomorrow, so a clear sentence tonight prevents four phone calls in the morning.*
4 · 2:00–2:40 Consequence	Saved as "not fit — optimise potassium". The case card across the theatre list and the readiness board updates immediately.	*Save, and it is everywhere at once. The surgical team see it, the theatre readiness board sees it, and the case stops pretending it is ready. Nobody has to be telephoned and nobody has to remember. That is the entire point of doing it here instead of writing it in the folder.*
5 · 2:40–3:10 Emergencies	Emergency alert arriving on the phone; the emergency pre-anaesthetic assessment opening, shorter, with the essentials only.	*Emergencies reach you the same way but faster: an alert on your phone, and a shorter assessment built for the corridor. Fill in what you have. An emergency review with three fields completed is worth more than a perfect one that never gets written.*
6 · 3:10–3:30 Close	Evening ward round with a phone; then a calm 08:00 start. Presenter.	*One thing to do tomorrow: open the board at six in the evening, not at eight in the morning.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

```
V10 · The anaesthetist board

SHOT 01
IMAGE:     08:10, theatre door. An anaesthetist meets a patient for the first time; the observation chart shows a potassium of 6.9. The case is stood down.
MOTION:    Slow push in from a wide establishing frame
NARRATION: He is meeting this patient for the first time at the theatre door. Her potassium is six point nine. The case is now cancelled, the list has lost two hours, and every one of those facts was available last night.

SHOT 02
IMAGE:     Anaesthetist Board on a phone: tomorrow's cases grouped by theatre, each showing patient, procedure, anaesthesia type, and a coloured review status.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: The Anaesthetist Board is your list, not the surgeon's. Tomorrow's cases, grouped by theatre, each showing the operation, the planned anaesthetic and whether it has been reviewed. Open it the evening before. Twenty minutes with this screen is the difference between a list that runs and a morning spent apologising.

SHOT 03
IMAGE:     A case opened. Pre-op review form: airway assessment, ASA grade, comorbidities, current medication, the lab results already entered by the team, and a fitness decision with a free-text note.
MOTION:    Hold steady on the screen, no camera move; highlight the tap target a beat before the finger arrives
NARRATION: Tap a case for the pre-operative review. The bloods the surgical team entered are already here — you are not chasing them. Add the airway assessment, the ASA grade, the comorbidities and the drugs she is on. Then the decision: fit, fit with conditions, or not fit. Write the reason. That note is read by the surgeon, the theatre coordinator and whoever is on tomorrow, so a clear sentence tonight prevents four phone calls in the morning.

SHOT 04
IMAGE:     Saved as "not fit — optimise potassium". The case card across the theatre list and the readiness board updates immediately.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Save, and it is everywhere at once. The surgical team see it, the theatre readiness board sees it, and the case stops pretending it is ready. Nobody has to be telephoned and nobody has to remember. That is the entire point of doing it here instead of writing it in the folder.

SHOT 05
IMAGE:     Emergency alert arriving on the phone; the emergency pre-anaesthetic assessment opening, shorter, with the essentials only.
MOTION:    Slow push in
NARRATION: Emergencies reach you the same way but faster: an alert on your phone, and a shorter assessment built for the corridor. Fill in what you have. An emergency review with three fields completed is worth more than a perfect one that never gets written.

SHOT 06
IMAGE:     Evening ward round with a phone; then a calm 08:00 start. Presenter.
MOTION:    Slow pull back, settling as the line lands
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NARRATION: One thing to do tomorrow: open the board at six in the evening, not at eight in the morning.

VIDEO 10 - THE ANAESTHETIST BOARD (3 min 30 s)
SCENES: 1 (0:00-0:20) 08:10 at the theatre door, chart showing potassium 6.9, case stood down. 2 (0:20-1:05) Anaesthetist Board on a phone; tomorrow grouped by theatre with review status colours. 3 (1:05-2:00) Pre-op review form: airway, ASA, comorbidities, medication, pre-filled labs, fitness decision with note. 4 (2:00-2:40) Saved as "not fit - optimise potassium"; theatre list and readiness board updating. 5 (2:40-3:10) Emergency alert; shorter emergency pre-anaesthetic assessment. 6 (3:10-3:30) Evening ward round with a phone; calm 08:00 start.
NARRATION: paste the narration column, in order.

V11 · Setup and intra-operative monitoring

Anaesthetist | Anaesthetic technician | 3 min 30 s

THEY WILL BE ABLE TO

- Complete the anaesthesia setup check before the patient arrives.
- Start monitoring and record observations through the case.
- Record drugs given so pharmacy reconciliation is automatic.

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:18 Hook	Empty theatre before a list. A technician checking a machine; the suction is not connected.	*Seven forty in the morning. Nobody is in the room yet, and the suction is not connected. This is the last five minutes in which that is a small problem.*
2 · 0:18–1:05 Setup	Anesthesia Setup screen: machine check, circuit, suction, airway trolley, drugs drawn, monitoring leads. Each ticked in turn; the theatre readiness indicator turns green.	*Anesthesia Setup is the pre-list check, and it is a list rather than a memory. Machine, circuit, suction, airway trolley, drugs drawn up, monitoring. Tick each as you do it — not before. When the last one is ticked, this theatre shows as anaesthetically ready on the readiness board, and the coordinator stops asking.*
3 · 1:05–2:05 Monitoring	Initialise monitoring for a case. A timeline appears. Observations entered: blood pressure, pulse, saturation, temperature. Each appears as a point on the chart.	*When the case starts, initialise monitoring. You get a timeline for this patient. Enter observations as you take them — pressure, pulse, saturation, temperature — and they build a chart as the case runs. If the connection drops, keep recording. It is held on the machine and sent when the signal returns; nothing you enter is lost.*
4 · 2:05–2:50 Drugs	Drug administration entries added to the same timeline: agent, dose, time. Then a pharmacy view showing the same entries reconciled.	*Record the drugs on the same timeline: what, how much, when. Two things follow. The anaesthetic record writes itself, so there is nothing to reconstruct afterwards. And the pharmacy sees what was actually used against what was issued, which is how the controlled-drug register stops being an argument at the end of the month.*
5 · 2:50–3:20 Handover	End of case. The record closes and appears in the recovery nurse's handover screen.	*At the end, close the record. It travels straight to recovery, so the nurse receiving your patient already has the anaesthetic, the drugs, the losses and your instructions before the trolley arrives.*
6 · 3:20–3:30 Close	Presenter.	*One thing to do tomorrow: tick the setup list as you do it, not from memory at the end.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

```
V11 · Setup and intra-operative monitoring

SHOT 01
IMAGE:     Empty theatre before a list. A technician checking a machine; the suction is not
connected.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Seven forty in the morning. Nobody is in the room yet, and the suction is not
connected. This is the last five minutes in which that is a small problem.

SHOT 02
IMAGE:     Anesthesia Setup screen: machine check, circuit, suction, airway trolley, drugs
drawn, monitoring leads. Each ticked in turn; the theatre readiness indicator turns green.
MOTION:    Hold steady on the screen, no camera move; highlight the tap target a beat before
the finger arrives
NARRATION: Anesthesia Setup is the pre-list check, and it is a list rather than a memory.
Machine, circuit, suction, airway trolley, drugs drawn up, monitoring. Tick each as you do it
— not before. When the last one is ticked, this theatre shows as anaesthetically ready on the
readiness board, and the coordinator stops asking.

SHOT 03
IMAGE:     Initialise monitoring for a case. A timeline appears. Observations entered: blood
pressure, pulse, saturation, temperature. Each appears as a point on the chart.
MOTION:    Slow pan left to right, following each element as it appears
NARRATION: When the case starts, initialise monitoring. You get a timeline for this patient.
Enter observations as you take them — pressure, pulse, saturation, temperature — and they
build a chart as the case runs. If the connection drops, keep recording. It is held on the
machine and sent when the signal returns; nothing you enter is lost.

SHOT 04
IMAGE:     Drug administration entries added to the same timeline: agent, dose, time. Then a
pharmacy view showing the same entries reconciled.
MOTION:    Slow pan left to right, following each element as it appears
NARRATION: Record the drugs on the same timeline: what, how much, when. Two things follow.
The anaesthetic record writes itself, so there is nothing to reconstruct afterwards. And the
pharmacy sees what was actually used against what was issued, which is how the controlled-
drug register stops being an argument at the end of the month.

SHOT 05
IMAGE:     End of case. The record closes and appears in the recovery nurse's handover
screen.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: At the end, close the record. It travels straight to recovery, so the nurse
receiving your patient already has the anaesthetic, the drugs, the losses and your
instructions before the trolley arrives.

SHOT 06
IMAGE:     Presenter.
MOTION:    Slow pull back, settling as the line lands
NARRATION: One thing to do tomorrow: tick the setup list as you do it, not from memory at the
end.
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VIDEO 11 - SETUP AND INTRA-OPERATIVE MONITORING (3 min 30 s)
SCENES: 1 (0:00-0:18) Empty theatre, technician finds suction not connected. 2 (0:18-1:05) Anesthesia Setup checklist ticked item by item; readiness indicator turning green. 3 (1:05-2:05) Monitoring initialised; observations entered building a live chart. 4 (2:05-2:50) Drug entries on the same timeline; pharmacy reconciliation view. 5 (2:50-3:20) Record closed and appearing in recovery handover. 6 (3:20-3:30) Presenter close.
NARRATION: paste the narration column, in order.

V12 · Coverage and the roster

Consultant anaesthetist | Anaesthesia rota-holder | 3 min

THE RULE THIS VIDEO EXISTS TO TEACH

An elective case is covered by the anaesthetist rostered to *its own specialty*, on the elective roster, or by nobody. The on-call team never silently covers a planned list. If nobody is rostered, the case reads **"Anaesthetist: not yet assigned"** — and that is a gap somebody must fill, not an error.

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:25 Hook	A surgery list where every single case, across eight theatres, names the same anaesthetist.	*Fifteen cases. Eight theatres. One anaesthetist's name on every one of them. He is not doing fifteen lists — he is the on-call consultant, and the elective roster for that day had not been published. The list looked staffed. It was not.*
2 · 0:25–1:10 How cover is decided	Animation: a roster row with a specialty against a name, matching to a case with the same specialty. Then a case with no matching row, showing "not yet assigned" in amber.	*Here is how the app decides. On the elective roster each anaesthetist is recorded against a surgical specialty for the day. A case takes the person rostered to its own specialty. If nobody is rostered to it, the case says, plainly, not yet assigned. It will not quietly borrow the on-call consultant, and it will not borrow a colleague rostered to a different specialty either — that is the same mistake wearing a more convincing name.*
3 · 1:10–1:55 Coverage screen	Anaesthetist Coverage: specialties down the left, tomorrow's rostered names against them, uncovered specialties highlighted amber with their case counts.	*Anaesthetist Coverage is the screen that shows you this before it hurts. Every specialty operating tomorrow, who is covering it, and in amber, the ones with cases and nobody assigned. Open it the afternoon before. An amber row is roughly two minutes of work now, or a stood-down eye list at nine in the morning.*
4 · 1:55–2:35 Assigning	An amber specialty tapped; a list of available anaesthetists; one selected; the row turns green and the cases update.	*Tap an amber row and assign somebody. The cases update immediately. And when the roster itself is published, cases already booked take their proper anaesthetist automatically — you do not go back through them one at a time.*
5 · 2:35–3:00 Close	A coverage screen with every row green. Presenter.	*On call means covering the unplanned. It has never meant covering everybody's elective list. One thing to do tomorrow: publish the elective roster before the cases are booked, not after.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

```
V12 · Coverage and the roster

SHOT 01
IMAGE:     A surgery list where every single case, across eight theatres, names the same anaesthetist.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Fifteen cases. Eight theatres. One anaesthetist's name on every one of them. He is not doing fifteen lists — he is the on-call consultant, and the elective roster for that day had not been published. The list looked staffed. It was not.

SHOT 02
IMAGE:     Animation: a roster row with a specialty against a name, matching to a case with the same specialty. Then a case with no matching row, showing "not yet assigned" in amber.
MOTION:    Slow pan left to right, following each element as it appears
NARRATION: Here is how the app decides. On the elective roster each anaesthetist is recorded against a surgical specialty for the day. A case takes the person rostered to its own specialty. If nobody is rostered to it, the case says, plainly, not yet assigned. It will not quietly borrow the on-call consultant, and it will not borrow a colleague rostered to a different specialty either — that is the same mistake wearing a more convincing name.

SHOT 03
IMAGE:     Anaesthetist Coverage: specialties down the left, tomorrow's rostered names against them, uncovered specialties highlighted amber with their case counts.
MOTION:    Slow push in
NARRATION: Anaesthetist Coverage is the screen that shows you this before it hurts. Every specialty operating tomorrow, who is covering it, and in amber, the ones with cases and nobody assigned. Open it the afternoon before. An amber row is roughly two minutes of work now, or a stood-down eye list at nine in the morning.

SHOT 04
IMAGE:     An amber specialty tapped; a list of available anaesthetists; one selected; the row turns green and the cases update.
MOTION:    Hold steady on the screen, no camera move; highlight the tap target a beat before the finger arrives
NARRATION: Tap an amber row and assign somebody. The cases update immediately. And when the roster itself is published, cases already booked take their proper anaesthetist automatically — you do not go back through them one at a time.

SHOT 05
IMAGE:     A coverage screen with every row green. Presenter.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: On call means covering the unplanned. It has never meant covering everybody's elective list. One thing to do tomorrow: publish the elective roster before the cases are booked, not after.
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VIDEO 12 - COVERAGE AND THE ROSTER (3 min)
SCENES: 1 (0:00-0:25) A surgery list where all fifteen cases across eight theatres name one anaesthetist. 2 (0:25-1:10) Animation of a roster row matching a case by specialty; then a case with no match showing amber "not yet assigned". 3 (1:10-1:55) Anaesthetist Coverage screen; uncovered specialties amber with case counts. 4 (1:55-2:35) Amber row tapped, anaesthetist selected, row turns green, cases update. 5 (2:35-3:00) All-green coverage screen; presenter
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close.
NARRATION: paste the narration column, in order.
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V13 · The anaesthetic technician

Anaesthetic technician | 2 min 30 s

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:18 Hook	A technician carrying a cylinder down a corridor, being stopped twice to answer questions.	*He has been stopped twice before he reached the door. Which theatre is he on today? Is Theatre Three's machine back? Both answers are on his phone.*
2 · 0:18–1:00 Your theatre	Technician Coverage screen: theatres listed, each with the technician assigned for the day; the viewer's own theatre highlighted.	*Technician Coverage shows which theatre you are on, and who is on the others. You are rostered to a theatre rather than a specialty, because your work follows the room. If your name is not there, the roster has not been published — say so early, not at half past seven.*
3 · 1:00–1:45 Machine and equipment	Anesthesia Setup for the assigned theatre; then Equipment Checkout showing a ventilator booked out and returned.	*Two screens carry your day. Anesthesia Setup is the machine and airway check for your room. Equipment Checkout is anything that moves between theatres — book it out when you take it and back in when it returns. The point is not bureaucracy. It is that when a machine is needed urgently, somebody can see which room it is in instead of opening six doors.*
4 · 1:45–2:20 Faults	Fault Alerts: raising a fault on a vaporiser, with a photograph attached. Biomedical engineering receives it.	*If something is broken, raise it in Fault Alerts and photograph it. It goes straight to biomedical engineering with the theatre, the machine and the time. A fault reported this way gets fixed. A fault mentioned in a corridor gets forgotten by the afternoon.*
5 · 2:20–2:30 Close	Presenter.	*One thing to do tomorrow: check Technician Coverage before you leave the changing room.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

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V13 · The anaesthetic technician

SHOT 01
IMAGE:     A technician carrying a cylinder down a corridor, being stopped twice to answer questions.
MOTION:    Slow dolly, travelling with the subject at walking pace
NARRATION: He has been stopped twice before he reached the door. Which theatre is he on today? Is Theatre Three's machine back? Both answers are on his phone.

SHOT 02
IMAGE:     Technician Coverage screen: theatres listed, each with the technician assigned for the day; the viewer's own theatre highlighted.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Technician Coverage shows which theatre you are on, and who is on the others. You are rostered to a theatre rather than a specialty, because your work follows the room. If your name is not there, the roster has not been published — say so early, not at half past seven.

SHOT 03
IMAGE:     Anesthesia Setup for the assigned theatre; then Equipment Checkout showing a ventilator booked out and returned.
MOTION:    Gentle handheld, almost still — observational, not restless
NARRATION: Two screens carry your day. Anesthesia Setup is the machine and airway check for your room. Equipment Checkout is anything that moves between theatres — book it out when you take it and back in when it returns. The point is not bureaucracy. It is that when a machine is needed urgently, somebody can see which room it is in instead of opening six doors.

SHOT 04
IMAGE:     Fault Alerts: raising a fault on a vaporiser, with a photograph attached. Biomedical engineering receives it.
MOTION:    Slow push in
NARRATION: If something is broken, raise it in Fault Alerts and photograph it. It goes straight to biomedical engineering with the theatre, the machine and the time. A fault reported this way gets fixed. A fault mentioned in a corridor gets forgotten by the afternoon.

SHOT 05
IMAGE:     Presenter.
MOTION:    Slow pull back, settling as the line lands
NARRATION: One thing to do tomorrow: check Technician Coverage before you leave the changing room.
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VIDEO 13 - THE ANAESTHETIC TECHNICIAN (2 min 30 s)
SCENES: 1 (0:00-0:18) Technician carrying a cylinder, stopped twice with questions. 2 (0:18-1:00) Technician Coverage screen with theatres and assigned technicians, own theatre highlighted. 3 (1:00-1:45) Anesthesia Setup for that theatre; Equipment Checkout with a ventilator booked out and returned. 4 (1:45-2:20) Fault Alerts: raising a vaporiser fault with a photograph. 5 (2:20-2:30) Presenter close.
NARRATION: paste the narration column, in order.
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Part 4 — Nursing

Scrub nurses, circulating nurses, reception, holding area and recovery. Nursing staff touch more screens in this application than any other group, and they are the ones who notice first when something is wrong.

V14 · Theatre readiness

Scrub nurse | Theatre coordinator | 3 min 30 s

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:20 Hook	A coordinator with a clipboard walking into four theatres in turn, asking the same question in each.	*Four rooms, the same question in each, twenty minutes gone, and by the time she reaches the fourth the answer from the first has already changed. There is one screen that answers it for every room at once.*
2 · 0:20–1:10 The board	Theatre Readiness: every theatre as a row, with columns for anaesthesia, equipment, packs, sterile supply, cleaning and staff, each a coloured state.	*Theatre Readiness is every room in the suite, side by side. Anaesthesia, equipment, packs, sterile supply, cleaning, staff. Green means somebody has confirmed it. Amber means it is waiting on someone. Red means it is a problem now. Nobody fills this in as a form — it builds itself from what each department has already recorded, which is why the pack list and the CSSD screens matter so much.*
3 · 1:10–2:00 Case pack readiness	Case Pack Readiness: tomorrow's cases with their consumable and drug packs, showing which are made up, which are outstanding.	*Case Pack Readiness is the same idea for the lists rather than the rooms. Every case tomorrow, and whether its pack exists yet. Look at this the evening before. A case with no pack at six in the evening is solvable. The same case at half past eight in the morning is a cancellation.*
4 · 2:00–2:45 Team check-in	Team Check-in on a phone. Staff tapping in for their theatre; the readiness board's staff column turning green.	*Team Check-in is how the board knows people are actually in the room. Tap in when you arrive at your theatre. It takes two seconds and it replaces the coordinator's walk. It is also the fairest record you have of who was where, on a day somebody claims a theatre was unstaffed.*
5 · 2:45–3:20 Milestones	Record Milestones screen; large buttons: patient sent for, patient arrived, anaesthesia started, knife to skin, closing, patient to recovery. One tapped, with the time recorded automatically.	*Through the day, Record Milestones. Big buttons, one tap, the time recorded for you. Sent for, arrived, anaesthesia started, knife to skin, out to recovery. These are what every report and every delay conversation is built from later. Tap them as they happen. Filled in from memory at four in the afternoon, they are fiction.*
6 · 3:20–3:30 Close	Board all green; presenter.	*One thing to do tomorrow: open Case Pack Readiness at six in the evening.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

```
V14 · Theatre readiness

SHOT 01
IMAGE:     A coordinator with a clipboard walking into four theatres in turn, asking the same question in each.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Four rooms, the same question in each, twenty minutes gone, and by the time she reaches the fourth the answer from the first has already changed. There is one screen that answers it for every room at once.

SHOT 02
IMAGE:     Theatre Readiness: every theatre as a row, with columns for anaesthesia, equipment, packs, sterile supply, cleaning and staff, each a coloured state.
MOTION:    Slow pan left to right, following each element as it appears
NARRATION: Theatre Readiness is every room in the suite, side by side. Anaesthesia, equipment, packs, sterile supply, cleaning, staff. Green means somebody has confirmed it. Amber means it is waiting on someone. Red means it is a problem now. Nobody fills this in as a form — it builds itself from what each department has already recorded, which is why the pack list and the CSSD screens matter so much.

SHOT 03
IMAGE:     Case Pack Readiness: tomorrow's cases with their consumable and drug packs, showing which are made up, which are outstanding.
MOTION:    Slow push in
NARRATION: Case Pack Readiness is the same idea for the lists rather than the rooms. Every case tomorrow, and whether its pack exists yet. Look at this the evening before. A case with no pack at six in the evening is solvable. The same case at half past eight in the morning is a cancellation.

SHOT 04
IMAGE:     Team Check-in on a phone. Staff tapping in for their theatre; the readiness board's staff column turning green.
MOTION:    Hold steady on the screen, no camera move; highlight the tap target a beat before the finger arrives
NARRATION: Team Check-in is how the board knows people are actually in the room. Tap in when you arrive at your theatre. It takes two seconds and it replaces the coordinator's walk. It is also the fairest record you have of who was where, on a day somebody claims a theatre was unstaffed.

SHOT 05
IMAGE:     Record Milestones screen; large buttons: patient sent for, patient arrived, anaesthesia started, knife to skin, closing, patient to recovery. One tapped, with the time recorded automatically.
MOTION:    Hold steady on the screen, no camera move; highlight the tap target a beat before the finger arrives
NARRATION: Through the day, Record Milestones. Big buttons, one tap, the time recorded for you. Sent for, arrived, anaesthesia started, knife to skin, out to recovery. These are what every report and every delay conversation is built from later. Tap them as they happen. Filled in from memory at four in the afternoon, they are fiction.

SHOT 06
IMAGE:     Board all green; presenter.
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MOTION: Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: One thing to do tomorrow: open Case Pack Readiness at six in the evening.

VIDEO 14 - THEATRE READINESS (3 min 30 s)
SCENES: 1 (0:00-0:20) Coordinator with clipboard asking the same question in four theatres. 2 (0:20-1:10) Theatre Readiness board: theatres as rows, columns for anaesthesia, equipment, packs, sterile supply, cleaning, staff, colour coded. 3 (1:10-2:00) Case Pack Readiness listing tomorrow's cases and pack status. 4 (2:00-2:45) Team Check-in on a phone; staff column turning green. 5 (2:45-3:20) Record Milestones with large one-tap buttons and automatic times. 6 (3:20-3:30) All-green board; presenter.
NARRATION: paste the narration column, in order.

V15 · The WHO checklists

Scrub nurse | Whole theatre team | 2 min 30 s

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:20 Hook	A team pausing before incision. Everyone stops. One person reads aloud from a tablet.	*Eleven seconds. That is how long this pause takes, and it is the most evidence-backed eleven seconds in surgery. The app does not replace it. It records that it happened.*
2 · 0:20–1:15 Three points	Checklists screen showing the three stages: sign in before anaesthesia, time out before incision, sign out before leaving. Sign-in being completed on a tablet.	*Three points in every case. Sign in, before anaesthesia: identity, site, consent, allergies, airway. Time out, before incision: everyone introduces themselves and the critical steps are stated. Sign out, before the patient leaves: counts, specimens, and what recovery needs to know. Each is a short list on the screen, ticked as it is spoken.*
3 · 1:15–2:00 Speak it	Team out loud; the person with the tablet ticking as answers come back, not before.	*One rule decides whether this is worth doing. Tick as the answer is spoken aloud, by the person responsible for it — not in advance, and not by one person reading to themselves. A checklist ticked silently is a checklist that has caught nothing. The value is entirely in the theatre hearing the answer.*
4 · 2:00–2:30 Close	Sign-out completing; count confirmed; presenter.	*One thing to do tomorrow: at time out, say your own name and your own role out loud, even if everyone knows you. It gives the newest person in the room permission to speak later, and that is the whole reason it exists.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

```
V15 · The WHO checklists

SHOT 01
IMAGE:     A team pausing before incision. Everyone stops. One person reads aloud from a
tablet.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Eleven seconds. That is how long this pause takes, and it is the most evidence-
backed eleven seconds in surgery. The app does not replace it. It records that it happened.

SHOT 02
IMAGE:     Checklists screen showing the three stages: sign in before anaesthesia, time out
before incision, sign out before leaving. Sign-in being completed on a tablet.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Three points in every case. Sign in, before anaesthesia: identity, site, consent,
allergies, airway. Time out, before incision: everyone introduces themselves and the critical
steps are stated. Sign out, before the patient leaves: counts, specimens, and what recovery
needs to know. Each is a short list on the screen, ticked as it is spoken.

SHOT 03
IMAGE:     Team out loud; the person with the tablet ticking as answers come back, not
before.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: One rule decides whether this is worth doing. Tick as the answer is spoken aloud,
by the person responsible for it — not in advance, and not by one person reading to
themselves. A checklist ticked silently is a checklist that has caught nothing. The value is
entirely in the theatre hearing the answer.

SHOT 04
IMAGE:     Sign-out completing; count confirmed; presenter.
MOTION:    Slow pull back, settling as the line lands
NARRATION: One thing to do tomorrow: at time out, say your own name and your own role out
loud, even if everyone knows you. It gives the newest person in the room permission to speak
later, and that is the whole reason it exists.
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VIDEO 15 - THE WHO CHECKLISTS (2 min 30 s)
SCENES: 1 (0:00-0:20) Team pausing before incision; one person reading from a tablet. 2 (0:20-
1:15) Checklists screen with sign in, time out, sign out; sign-in completed. 3 (1:15-2:00) Team
answering aloud while the tablet is ticked in response. 4 (2:00-2:30) Sign-out completing with
count confirmed; presenter.
NARRATION: paste the narration column, in order.
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V16 · Reception and holding area

Theatre reception | Holding area nurse | 3 min

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:20 Hook	A patient on a trolley in the holding corridor, alone, no notes. A nurse looking through three folders.	*She arrived nine minutes ago. Nobody in this corridor knows which theatre she is for, and her folder is still on the ward. This is the point in the journey where patients get lost.*
2 · 0:20–1:05 Reception	Theatre Reception on a tablet: expected patients today; one marked arrived; identity and folder confirmed against the screen.	*Theatre Reception lists everyone expected today. When the patient reaches the door, find her and mark her arrived. That single tap tells the theatre she is in the building, stops the send-for reminder repeating, and starts the clock that tells you honestly how long she waited.*
3 · 1:05–2:00 Holding assessment	Holding Area assessment: identity, site marked, fasting time, consent sighted, jewellery and prosthetics, allergies, observations. Each confirmed.	*In the holding area, the last check before the doors. Identity, the marked site, when she last ate, consent sighted, jewellery, allergies, and a set of observations. This is the final chance to catch something, and it is deliberately the same list every time so that nothing depends on who is on duty.*
4 · 2:00–2:40 When something is wrong	Fasting time flagged as too short; the case flagged; the anaesthetist's phone alerting.	*If something does not add up, record it as it is. Do not round it, do not assume somebody else has noticed. Flag it here and the anaesthetist knows within seconds, while the patient is still outside the theatre and there are still choices.*
5 · 2:40–3:00 Close	Doors opening, patient going through calmly; presenter.	*One thing to do tomorrow: mark arrival the moment the trolley stops, not when you get a free minute.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

```
V16 · Reception and holding area

SHOT 01
IMAGE:     A patient on a trolley in the holding corridor, alone, no notes. A nurse looking through three folders.
MOTION:    Slow dolly, travelling with the subject at walking pace
NARRATION: She arrived nine minutes ago. Nobody in this corridor knows which theatre she is for, and her folder is still on the ward. This is the point in the journey where patients get lost.

SHOT 02
IMAGE:     Theatre Reception on a tablet: expected patients today; one marked arrived; identity and folder confirmed against the screen.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Theatre Reception lists everyone expected today. When the patient reaches the door, find her and mark her arrived. That single tap tells the theatre she is in the building, stops the send-for reminder repeating, and starts the clock that tells you honestly how long she waited.

SHOT 03
IMAGE:     Holding Area assessment: identity, site marked, fasting time, consent sighted, jewellery and prosthetics, allergies, observations. Each confirmed.
MOTION:    Slow push in
NARRATION: In the holding area, the last check before the doors. Identity, the marked site, when she last ate, consent sighted, jewellery, allergies, and a set of observations. This is the final chance to catch something, and it is deliberately the same list every time so that nothing depends on who is on duty.

SHOT 04
IMAGE:     Fasting time flagged as too short; the case flagged; the anaesthetist's phone alerting.
MOTION:    Slow push in
NARRATION: If something does not add up, record it as it is. Do not round it, do not assume somebody else has noticed. Flag it here and the anaesthetist knows within seconds, while the patient is still outside the theatre and there are still choices.

SHOT 05
IMAGE:     Doors opening, patient going through calmly; presenter.
MOTION:    Slow pull back, settling as the line lands
NARRATION: One thing to do tomorrow: mark arrival the moment the trolley stops, not when you get a free minute.
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VIDEO 16 - RECEPTION AND HOLDING AREA (3 min)
SCENES: 1 (0:00-0:20) Patient alone on a trolley in a corridor; nurse searching three folders. 2 (0:20-1:05) Theatre Reception on a tablet; patient marked arrived. 3 (1:05-2:00) Holding Area assessment: identity, marked site, fasting, consent sighted, jewellery, allergies, observations. 4 (2:00-2:40) Short fasting time flagged; anaesthetist's phone alerting. 5 (2:40-3:00) Doors opening calmly; presenter.
NARRATION: paste the narration column, in order.
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V17 · Calling for the patient

Theatre nurse | Porter | Ward nurse | 2 min 30 s

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:20 Hook	Theatre ready, team scrubbed, empty table. Cut to a ward where the patient sits fully dressed, nobody having asked for her.	*The theatre has been ready for twenty-five minutes. On the ward she is still in her own clothes, because nobody has asked for her. Neither room knows about the other.*
2 · 0:20–1:10 Call for patient	Call for Patient screen: today's cases in order; one tapped; a confirmation. Corridor speaker announcing by name and ward; the porter's phone buzzing.	*Call for Patient. Find the case and tap it. Three things happen at once: the radio announces her by name and ward, the porters are alerted on their phones, and the ward sees the request on their own screen. One tap replaces three phone calls, and the time is recorded, so if she takes forty minutes to arrive, that is visible rather than argued about.*
3 · 1:10–1:50 The hour before	An automatic reminder an hour before the scheduled start, announced over the radio and shown on the board.	*You also get help before you ask. About an hour before each case is due, the app announces it by itself, so the ward can start getting her ready. That reminder is why the first case of the afternoon is now on time more often than it used to be.*
4 · 1:50–2:20 Arrival	Patient arrives; reception marks arrival; the repeating announcement stops.	*When she arrives, reception marks it, and the announcement stops repeating. If you ever hear the same call over and over, it means nobody has marked the arrival — and that is worth fixing straight away, because an alert people learn to ignore is worse than no alert at all.*
5 · 2:20–2:30 Close	Presenter.	*One thing to do tomorrow: call for the next patient while the current one is still being closed.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

```
V17 · Calling for the patient

SHOT 01
IMAGE:     Theatre ready, team scrubbed, empty table. Cut to a ward where the patient sits fully dressed, nobody having asked for her.
MOTION:    Slow push in from a wide establishing frame
NARRATION: The theatre has been ready for twenty-five minutes. On the ward she is still in her own clothes, because nobody has asked for her. Neither room knows about the other.

SHOT 02
IMAGE:     Call for Patient screen: today's cases in order; one tapped; a confirmation. Corridor speaker announcing by name and ward; the porter's phone buzzing.
MOTION:    Hold steady on the screen, no camera move; highlight the tap target a beat before the finger arrives
NARRATION: Call for Patient. Find the case and tap it. Three things happen at once: the radio announces her by name and ward, the porters are alerted on their phones, and the ward sees the request on their own screen. One tap replaces three phone calls, and the time is recorded, so if she takes forty minutes to arrive, that is visible rather than argued about.

SHOT 03
IMAGE:     An automatic reminder an hour before the scheduled start, announced over the radio and shown on the board.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: You also get help before you ask. About an hour before each case is due, the app announces it by itself, so the ward can start getting her ready. That reminder is why the first case of the afternoon is now on time more often than it used to be.

SHOT 04
IMAGE:     Patient arrives; reception marks arrival; the repeating announcement stops.
MOTION:    Slow push in
NARRATION: When she arrives, reception marks it, and the announcement stops repeating. If you ever hear the same call over and over, it means nobody has marked the arrival — and that is worth fixing straight away, because an alert people learn to ignore is worse than no alert at all.

SHOT 05
IMAGE:     Presenter.
MOTION:    Slow pull back, settling as the line lands
NARRATION: One thing to do tomorrow: call for the next patient while the current one is still being closed.
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VIDEO 17 - CALLING FOR THE PATIENT (2 min 30 s)
SCENES: 1 (0:00-0:20) Ready theatre with an empty table; ward with the patient still dressed. 2 (0:20-1:10) Call for Patient tapped; corridor speaker announcing name and ward; porter's phone buzzing; ward screen updating. 3 (1:10-1:50) Automatic reminder an hour before the scheduled start. 4 (1:50-2:20) Arrival marked; repeating announcement stops. 5 (2:20-2:30) Presenter.
NARRATION: paste the narration column, in order.
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V18 · Nurse handover

Scrub nurse | Circulating nurse | 2 min 30 s

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:20 Hook	Shift change. Two nurses talking quickly in a doorway while a case is still running.	*Forty seconds in a doorway, with a case still running behind them. Everything she has learned in six hours has to cross that gap. Most of it will not.*
2 · 0:20–1:20 The handover	Nurse Handover screen: theatre, cases done, cases remaining, equipment faults, stock used and low, patients of concern, notes for the next shift.	*Nurse Handover gives that conversation a shape. What has been done, what is still to come, what broke, what has run low, and which patient you are worried about. Write it as the shift runs, not at the end — a line at a time, when it happens. It takes ten seconds each and it means the person following you starts where you finished instead of where you started.*
3 · 1:20–2:05 Receiving	Incoming nurse opening the handover on a phone in the changing room, before entering the theatre.	*And if you are coming on, read it before you walk in. Two minutes in the changing room, and you enter the room already knowing that the diathermy is intermittent and that the second case has a difficult airway. That is a different shift from the one that begins with a whispered handover in a doorway.*
4 · 2:05–2:30 Close	Calm shift change; presenter.	*One thing to do tomorrow: add one line to the handover as soon as anything goes wrong, while you are still annoyed about it.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

```
V18 · Nurse handover

SHOT 01
IMAGE:     Shift change. Two nurses talking quickly in a doorway while a case is still running.
MOTION:    Slow push in
NARRATION: Forty seconds in a doorway, with a case still running behind them. Everything she has learned in six hours has to cross that gap. Most of it will not.

SHOT 02
IMAGE:     Nurse Handover screen: theatre, cases done, cases remaining, equipment faults, stock used and low, patients of concern, notes for the next shift.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Nurse Handover gives that conversation a shape. What has been done, what is still to come, what broke, what has run low, and which patient you are worried about. Write it as the shift runs, not at the end — a line at a time, when it happens. It takes ten seconds each and it means the person following you starts where you finished instead of where you started.

SHOT 03
IMAGE:     Incoming nurse opening the handover on a phone in the changing room, before entering the theatre.
MOTION:    Gentle handheld, almost still — observational, not restless
NARRATION: And if you are coming on, read it before you walk in. Two minutes in the changing room, and you enter the room already knowing that the diathermy is intermittent and that the second case has a difficult airway. That is a different shift from the one that begins with a whispered handover in a doorway.

SHOT 04
IMAGE:     Calm shift change; presenter.
MOTION:    Slow pull back, settling as the line lands
NARRATION: One thing to do tomorrow: add one line to the handover as soon as anything goes wrong, while you are still annoyed about it.
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VIDEO 18 - NURSE HANDOVER (2 min 30 s)
SCENES: 1 (0:00-0:20) Two nurses handing over quickly in a doorway during a running case. 2 (0:20-1:20) Nurse Handover screen: cases done and remaining, faults, stock, patients of concern, notes. 3 (1:20-2:05) Incoming nurse reading it on a phone in the changing room. 4 (2:05-2:30) Calm shift change; presenter.
NARRATION: paste the narration column, in order.
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V19 · Recovery (PACU)

Recovery room nurse | 3 min 30 s

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:20 Hook	A patient wheeled into recovery. The nurse receiving has no notes, and the anaesthetist has already gone back in.	*She has thirty seconds of handover and no notes, and the anaesthetist is already back in theatre. She is about to look after somebody she knows nothing about.*
2 · 0:20–1:05 Arrival	PACU screen. The patient already listed, with the anaesthetic record, drugs given, losses, and the anaesthetist's instructions attached.	*Open PACU and she is already there. The anaesthetic that was used, the drugs given and when, blood loss, and what the anaesthetist wants watched. It arrived when the record was closed in theatre. You are not waiting for paper and you are not relying on a doorway conversation.*
3 · 1:05–2:00 Observations	Recording observations at intervals; the trend building; pain score and nausea recorded; a medication given and logged.	*Record observations as you take them and they build a trend rather than a page of numbers. Pain and nausea go here too, because they are the two things that decide how the patient remembers the whole operation. Anything you give is logged against her, so the ward sees exactly what she has had and when it is next due.*
4 · 2:00–2:50 Discharge	Discharge criteria scored; the score reaching threshold; destination confirmed; ward notified automatically.	*Discharge is a score, not a feeling. Work through the criteria — consciousness, breathing, circulation, pain, nausea, mobility. When she meets them, record the destination and the ward is told she is coming. If she does not meet them, that is recorded too, and it is the strongest thing you can hold when somebody is pressing you for the bay.*
5 · 2:50–3:20 Escalation	A deteriorating patient; an alert raised from the PACU screen; the anaesthetist's phone alerting.	*If a patient deteriorates, raise it from this screen. It alerts the anaesthetist directly rather than sending somebody to find one, and it timestamps the moment you were worried — which matters afterwards, for you.*
6 · 3:20–3:30 Close	Presenter.	*One thing to do tomorrow: open PACU before the trolley arrives, not after.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

```
V19 · Recovery (PACU)

SHOT 01
IMAGE:     A patient wheeled into recovery. The nurse receiving has no notes, and the anaesthetist has already gone back in.
MOTION:    Slow dolly, travelling with the subject at walking pace
NARRATION: She has thirty seconds of handover and no notes, and the anaesthetist is already back in theatre. She is about to look after somebody she knows nothing about.

SHOT 02
IMAGE:     PACU screen. The patient already listed, with the anaesthetic record, drugs given, losses, and the anaesthetist's instructions attached.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Open PACU and she is already there. The anaesthetic that was used, the drugs given and when, blood loss, and what the anaesthetist wants watched. It arrived when the record was closed in theatre. You are not waiting for paper and you are not relying on a doorway conversation.

SHOT 03
IMAGE:     Recording observations at intervals; the trend building; pain score and nausea recorded; a medication given and logged.
MOTION:    Slow push in
NARRATION: Record observations as you take them and they build a trend rather than a page of numbers. Pain and nausea go here too, because they are the two things that decide how the patient remembers the whole operation. Anything you give is logged against her, so the ward sees exactly what she has had and when it is next due.

SHOT 04
IMAGE:     Discharge criteria scored; the score reaching threshold; destination confirmed; ward notified automatically.
MOTION:    Gentle handheld, almost still — observational, not restless
NARRATION: Discharge is a score, not a feeling. Work through the criteria — consciousness, breathing, circulation, pain, nausea, mobility. When she meets them, record the destination and the ward is told she is coming. If she does not meet them, that is recorded too, and it is the strongest thing you can hold when somebody is pressing you for the bay.

SHOT 05
IMAGE:     A deteriorating patient; an alert raised from the PACU screen; the anaesthetist's phone alerting.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: If a patient deteriorates, raise it from this screen. It alerts the anaesthetist directly rather than sending somebody to find one, and it timestamps the moment you were worried — which matters afterwards, for you.

SHOT 06
IMAGE:     Presenter.
MOTION:    Slow pull back, settling as the line lands
NARRATION: One thing to do tomorrow: open PACU before the trolley arrives, not after.
```

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VIDEO 19 - RECOVERY (PACU) (3 min 30 s)
SCENES: 1 (0:00-0:20) Patient wheeled into recovery, nurse with no notes, anaesthetist leaving. 2
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(0:20-1:05) PACU screen already showing the patient with anaesthetic record, drugs, losses,
instructions. 3 (1:05-2:00) Observations recorded building a trend; pain and nausea; medication
logged. 4 (2:00-2:50) Discharge criteria scored; threshold met; destination set; ward notified. 5
(2:50-3:20) Deterioration; alert raised; anaesthetist's phone alerting. 6 (3:20-3:30) Presenter.
NARRATION: paste the narration column, in order.
```

Part 5 — Pharmacy, blood bank and laboratory

Three departments that receive work automatically the moment a case is booked, and whose answers decide whether the morning list runs.

V20 · Pharmacy and drug packs

Pharmacist | 3 min 30 s

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:20 Hook	Pharmacy window, 07:50. A house officer arrives out of breath with a handwritten list for a case starting at eight.	*Ten to eight, a handwritten list, and a case that starts in ten minutes. This used to be every morning. It does not have to be, because that list was written three weeks ago and nobody sent it to you.*
2 · 0:20–1:10 Work arrives	Pharmacy screen: tomorrow's cases, each with its drug and dressing list, patient, procedure and start time. A notification badge on the menu item.	*Open Pharmacy and the work is already here. Every case booked for tomorrow, with the drugs and dressings the surgeon asked for, the patient, the operation and the time it starts. It arrived the moment the case was booked — often days in advance. You are notified when a new one lands, so you are not refreshing a screen.*
3 · 1:10–2:00 Preparing	A case opened; items ticked as picked; one item marked unavailable with a note; status moves to packed.	*Work down the list, ticking as you pick. If something is not available, say so here rather than leaving a gap — mark it and write what you have instead. That single note appears on the surgeon's case and on the theatre readiness board tonight, which is the difference between a substitution agreed calmly in the evening and an argument at the theatre door.*
4 · 2:00–2:40 The patient code	A patient at the window handing over a card with a pharmacy code. The code typed in; the correct pack appears immediately.	*When the patient comes to the window she brings a code the surgeon gave her at booking. Type it in and her pack comes straight up — no folder number, no spelling her name three ways, no phone call to the ward. Encourage patients to bring it. It is the fastest thing at this counter.*
5 · 2:40–3:15 Approvals and tracking	Rx Approvals screen with a controlled item awaiting sign-off; then Med Tracking showing issued against administered for a completed case.	*Two more screens. Rx Approvals holds anything needing a pharmacist's sign-off before it goes. Med Tracking shows what was issued against what the anaesthetist recorded as given, case by case. That reconciliation used to be a monthly argument reconstructed from memory. It is now a screen you can open on the day.*
6 · 3:15–3:30 Close	Calm pharmacy at 07:50, packs already labelled and waiting. Presenter.	*One thing to do tomorrow: open Pharmacy at four in the afternoon and prepare the morning list before you go home.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

```
V20 · Pharmacy and drug packs

SHOT 01
IMAGE:     Pharmacy window, 07:50. A house officer arrives out of breath with a handwritten
list for a case starting at eight.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Ten to eight, a handwritten list, and a case that starts in ten minutes. This used
to be every morning. It does not have to be, because that list was written three weeks ago
and nobody sent it to you.

SHOT 02
IMAGE:     Pharmacy screen: tomorrow's cases, each with its drug and dressing list, patient,
procedure and start time. A notification badge on the menu item.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Open Pharmacy and the work is already here. Every case booked for tomorrow, with
the drugs and dressings the surgeon asked for, the patient, the operation and the time it
starts. It arrived the moment the case was booked — often days in advance. You are notified
when a new one lands, so you are not refreshing a screen.

SHOT 03
IMAGE:     A case opened; items ticked as picked; one item marked unavailable with a note;
status moves to packed.
MOTION:    Slow push in
NARRATION: Work down the list, ticking as you pick. If something is not available, say so
here rather than leaving a gap — mark it and write what you have instead. That single note
appears on the surgeon's case and on the theatre readiness board tonight, which is the
difference between a substitution agreed calmly in the evening and an argument at the theatre
door.

SHOT 04
IMAGE:     A patient at the window handing over a card with a pharmacy code. The code typed
in; the correct pack appears immediately.
MOTION:    Gentle handheld, almost still — observational, not restless
NARRATION: When the patient comes to the window she brings a code the surgeon gave her at
booking. Type it in and her pack comes straight up — no folder number, no spelling her name
three ways, no phone call to the ward. Encourage patients to bring it. It is the fastest
thing at this counter.

SHOT 05
IMAGE:     Rx Approvals screen with a controlled item awaiting sign-off; then Med Tracking
showing issued against administered for a completed case.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Two more screens. Rx Approvals holds anything needing a pharmacist's sign-off
before it goes. Med Tracking shows what was issued against what the anaesthetist recorded as
given, case by case. That reconciliation used to be a monthly argument reconstructed from
memory. It is now a screen you can open on the day.

SHOT 06
IMAGE:     Calm pharmacy at 07:50, packs already labelled and waiting. Presenter.
MOTION:    Slow pull back, settling as the line lands
NARRATION: One thing to do tomorrow: open Pharmacy at four in the afternoon and prepare the
morning list before you go home.
```

VIDEO 20 - PHARMACY AND DRUG PACKS (3 min 30 s)
SCENES: 1 (0:00-0:20) Pharmacy window at 07:50; breathless house officer with a handwritten list. 2 (0:20-1:10) Pharmacy screen: tomorrow's cases with drug lists, patient, procedure, start time; notification badge. 3 (1:10-2:00) A case opened; items ticked; one marked unavailable with a substitution note; status set to packed. 4 (2:00-2:40) Patient hands over a card with a pharmacy code; typed in; the pack appears. 5 (2:40-3:15) Rx Approvals with a controlled item; Med Tracking showing issued against administered. 6 (3:15-3:30) Calm pharmacy, packs labelled and waiting.
NARRATION: paste the narration column, in order.

V21 · Blood bank

Blood bank staff | 3 min

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:20 Hook	A theatre mid-case, surgeon asking for blood. A runner sent down a corridor. Clock ticking.	*The blood was requested at the moment it was needed. Cross-matching takes time that this patient does not have. The request should have reached you yesterday, and the system already knew.*
2 · 0:20–1:05 Requests	Blood Bank screen: requests listed with patient, folder number, group, units, the surgery date and whether it is an emergency.	*Every request carries the patient, the folder number, the group, how many units, and when the operation is. Emergency requests sit at the top and look different, because they are. Acknowledge a request as you pick it up — the theatre can then see that a human being has it, which stops the runner being sent.*
3 · 1:05–1:50 Cross-match	A request opened; cross-match recorded as completed; units reserved against the patient; status visible on the theatre board.	*Record the cross-match when it is done and reserve the units against that patient. The theatre readiness board updates, so the surgical team can see blood is ready without telephoning. If a group is unavailable or a unit is short, record that instead. An honest shortage seen at four in the afternoon can be solved. The same shortage discovered at incision cannot.*
4 · 1:50–2:35 Emergencies	An emergency request arriving with an alert; the same screen, fewer steps; units issued urgently and logged.	*Emergency requests alert you directly rather than waiting to be noticed. The process is the same, with fewer steps. Everything you issue is logged against the patient and the case, so the return, the traceability and the audit are done as a by-product of the work rather than as a second job at the end of the week.*
5 · 2:35–3:00 Close	Blood ready in a cool box before the list starts. Presenter.	*One thing to do tomorrow: acknowledge every request the moment you read it, even before you start work on it.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

```
V21 · Blood bank

SHOT 01
IMAGE:     A theatre mid-case, surgeon asking for blood. A runner sent down a corridor. Clock ticking.
MOTION:    Slow dolly, travelling with the subject at walking pace
NARRATION: The blood was requested at the moment it was needed. Cross-matching takes time that this patient does not have. The request should have reached you yesterday, and the system already knew.

SHOT 02
IMAGE:     Blood Bank screen: requests listed with patient, folder number, group, units, the surgery date and whether it is an emergency.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Every request carries the patient, the folder number, the group, how many units, and when the operation is. Emergency requests sit at the top and look different, because they are. Acknowledge a request as you pick it up — the theatre can then see that a human being has it, which stops the runner being sent.

SHOT 03
IMAGE:     A request opened; cross-match recorded as completed; units reserved against the patient; status visible on the theatre board.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Record the cross-match when it is done and reserve the units against that patient. The theatre readiness board updates, so the surgical team can see blood is ready without telephoning. If a group is unavailable or a unit is short, record that instead. An honest shortage seen at four in the afternoon can be solved. The same shortage discovered at incision cannot.

SHOT 04
IMAGE:     An emergency request arriving with an alert; the same screen, fewer steps; units issued urgently and logged.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Emergency requests alert you directly rather than waiting to be noticed. The process is the same, with fewer steps. Everything you issue is logged against the patient and the case, so the return, the traceability and the audit are done as a by-product of the work rather than as a second job at the end of the week.

SHOT 05
IMAGE:     Blood ready in a cool box before the list starts. Presenter.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: One thing to do tomorrow: acknowledge every request the moment you read it, even before you start work on it.
```

```
VIDEO 21 - BLOOD BANK (3 min)
SCENES: 1 (0:00-0:20) Mid-case request for blood; runner sent down a corridor; clock ticking. 2 (0:20-1:05) Blood Bank request list with patient, folder, group, units, surgery date, emergencies at top. 3 (1:05-1:50) Cross-match recorded; units reserved; theatre board updating. 4 (1:50-2:35) Emergency request arriving with an alert; urgent issue logged. 5 (2:35-3:00) Blood ready in a cool box before the list starts.
NARRATION: paste the narration column, in order.
```

V22 · Laboratory and emergency workup

Laboratory staff | Emergency lab scientist | 3 min

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:20 Hook	A result printed and sitting in a tray. Cut to a theatre where the same result is being asked for by phone.	*This result has existed for two hours. It is in a tray, twelve metres from a theatre that is currently telephoning to ask for it. The only thing missing is somebody typing it once.*
2 · 0:20–1:05 What is outstanding	Emergency Lab Workup screen: patients with outstanding investigations, showing which tests and how urgent, ordered by surgery time.	*Emergency Lab Workup shows the patients waiting on you, which tests, and when their operation is. It is ordered by how soon they are needed, not by when the sample arrived, which is a different order and usually a better one.*
3 · 1:05–2:00 Entering results	A patient opened; haemoglobin, potassium, sodium, creatinine entered with sample time; viral screen results set. Each one clears an outstanding item on the case.	*Enter the result and the time the sample was taken. Watch what happens to the case: each value you enter removes an outstanding item from the surgeon's list, the anaesthetist's board and the readiness screen at the same moment. One entry, three departments told. That is the whole return on the thirty seconds it takes.*
4 · 2:00–2:40 Critical values	A markedly abnormal potassium entered; the case flags; the anaesthetist and surgical team alerted.	*If a value is dangerous, entering it is the fastest way to raise the alarm. The case flags immediately and the anaesthetist is told, without you finding a phone number or hoping somebody checks the tray. You have done the most useful thing you can do with that number in the moment you know it.*
5 · 2:40–3:00 Close	Empty tray; presenter.	*One thing to do tomorrow: enter results as they are validated, rather than in a batch at the end of the shift.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

```
V22 · Laboratory and emergency workup

SHOT 01
IMAGE:     A result printed and sitting in a tray. Cut to a theatre where the same result is being asked for by phone.
MOTION:    Slow push in from a wide establishing frame
NARRATION: This result has existed for two hours. It is in a tray, twelve metres from a theatre that is currently telephoning to ask for it. The only thing missing is somebody typing it once.

SHOT 02
IMAGE:     Emergency Lab Workup screen: patients with outstanding investigations, showing which tests and how urgent, ordered by surgery time.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Emergency Lab Workup shows the patients waiting on you, which tests, and when their operation is. It is ordered by how soon they are needed, not by when the sample arrived, which is a different order and usually a better one.

SHOT 03
IMAGE:     A patient opened; haemoglobin, potassium, sodium, creatinine entered with sample time; viral screen results set. Each one clears an outstanding item on the case.
MOTION:    Hold steady on the screen, no camera move; highlight the tap target a beat before the finger arrives
NARRATION: Enter the result and the time the sample was taken. Watch what happens to the case: each value you enter removes an outstanding item from the surgeon's list, the anaesthetist's board and the readiness screen at the same moment. One entry, three departments told. That is the whole return on the thirty seconds it takes.

SHOT 04
IMAGE:     A markedly abnormal potassium entered; the case flags; the anaesthetist and surgical team alerted.
MOTION:    Slow push in
NARRATION: If a value is dangerous, entering it is the fastest way to raise the alarm. The case flags immediately and the anaesthetist is told, without you finding a phone number or hoping somebody checks the tray. You have done the most useful thing you can do with that number in the moment you know it.

SHOT 05
IMAGE:     Empty tray; presenter.
MOTION:    Slow pull back, settling as the line lands
NARRATION: One thing to do tomorrow: enter results as they are validated, rather than in a batch at the end of the shift.
```

```
VIDEO 22 - LABORATORY AND EMERGENCY WORKUP (3 min)
SCENES: 1 (0:00-0:20) A printed result in a tray; a theatre telephoning for the same result. 2 (0:20-1:05) Emergency Lab Workup listing patients with outstanding tests ordered by surgery time. 3 (1:05-2:00) Results entered with sample times; outstanding items clearing across three screens. 4 (2:00-2:40) A dangerous potassium entered; case flags; anaesthetist alerted. 5 (2:40-3:00) Empty tray; presenter.
NARRATION: paste the narration column, in order.
```

Part 6 — Sterile services, supply and logistics

The departments that make the list physically possible. None of them are clinical, and every one of them can stop a case.

V23 · CSSD

CSSD staff | CSSD supervisor | 3 min

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:20 Hook	A theatre waiting; a set opened and found to be the wrong one. The scrub nurse's face.	*The set is open. It is the wrong one, and now it is also unusable. Somewhere in the department the right one is sitting sterile on a shelf, and the only thing that failed was knowing.*
2 · 0:20–1:10 Inventory	CSSD Inventory: instrument sets with location, sterile status and expiry.	*CSSD Inventory is every set, where it is, whether it is sterile and when that expires. Keep it accurate as things move and it answers the question the theatre keeps ringing to ask. A set whose record says sterile but is actually in the washer is worse than no record at all, so record the move as you make it.*
3 · 1:10–2:00 Sterilisation cycles	Sterilization screen: a load created, contents listed, cycle parameters recorded, indicator result entered, load released.	*Every cycle is recorded: what went in, the parameters, the indicator result, and who released the load. That is your traceability. If a question is ever asked about an instrument used in a particular case — and one day it will be — this is the record that answers it in a minute instead of a week.*
4 · 2:00–2:40 Readiness	CSSD Readiness: tomorrow's cases against the sets they need, showing which are ready and which are still processing.	*CSSD Readiness turns tomorrow's operating list into your work list. The sets each case needs, and whether they will be ready. Look at it in the afternoon. A set that will not be ready by nine tomorrow is a phone call you can make today, to a surgeon who still has options.*
5 · 2:40–3:00 Close	Correct set delivered and opened. Presenter.	*One thing to do tomorrow: open CSSD Readiness at two in the afternoon and work backwards from the morning list.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

```
V23 · CSSD

SHOT 01
IMAGE:     A theatre waiting; a set opened and found to be the wrong one. The scrub nurse's face.
MOTION:    Slow push in from a wide establishing frame
NARRATION: The set is open. It is the wrong one, and now it is also unusable. Somewhere in the department the right one is sitting sterile on a shelf, and the only thing that failed was knowing.

SHOT 02
IMAGE:     CSSD Inventory: instrument sets with location, sterile status and expiry.
MOTION:    Slow push in
NARRATION: CSSD Inventory is every set, where it is, whether it is sterile and when that expires. Keep it accurate as things move and it answers the question the theatre keeps ringing to ask. A set whose record says sterile but is actually in the washer is worse than no record at all, so record the move as you make it.

SHOT 03
IMAGE:     Sterilization screen: a load created, contents listed, cycle parameters recorded, indicator result entered, load released.
MOTION:    Hold steady on the screen, no camera move; highlight the tap target a beat before the finger arrives
NARRATION: Every cycle is recorded: what went in, the parameters, the indicator result, and who released the load. That is your traceability. If a question is ever asked about an instrument used in a particular case — and one day it will be — this is the record that answers it in a minute instead of a week.

SHOT 04
IMAGE:     CSSD Readiness: tomorrow's cases against the sets they need, showing which are ready and which are still processing.
MOTION:    Slow push in
NARRATION: CSSD Readiness turns tomorrow's operating list into your work list. The sets each case needs, and whether they will be ready. Look at it in the afternoon. A set that will not be ready by nine tomorrow is a phone call you can make today, to a surgeon who still has options.

SHOT 05
IMAGE:     Correct set delivered and opened. Presenter.
MOTION:    Slow pull back, settling as the line lands
NARRATION: One thing to do tomorrow: open CSSD Readiness at two in the afternoon and work backwards from the morning list.
```

```
VIDEO 23 - CSSD (3 min)
SCENES: 1 (0:00-0:20) Wrong set opened in theatre; scrub nurse's reaction. 2 (0:20-1:10) CSSD Inventory: sets with location, sterile status, expiry. 3 (1:10-2:00) Sterilization: load created, contents, parameters, indicator result, release. 4 (2:00-2:40) CSSD Readiness matching tomorrow's cases to required sets. 5 (2:40-3:00) Correct set delivered and opened.
NARRATION: paste the narration column, in order.
```

V24 · Theatre store and supply

Theatre store keeper | Theatre supply unit | 3 min 30 s

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:20 Hook	A store with full shelves. A theatre cancelled for a missing item that is on a shelf in the next room, in a box nobody opened.	*That item is in this room. It has been for three weeks. The case was cancelled anyway, because the shelf knew and the system did not.*
2 · 0:20–1:05 Inventory and movements	Inventory screen: items, quantities, reorder levels, low-stock highlighted. A stock movement being recorded as items are issued.	*Inventory holds what you have, what the reorder level is, and highlights what is running low. The balance is not typed — it is calculated from movements. So record issues and receipts as they happen, and the number stays true by itself. Overwrite a balance by hand and you have hidden the very thing the screen exists to show.*
3 · 1:05–2:00 Requests from theatre	Theatre Supply Unit: consumable requests from booked cases, grouped by case and date, with the standard theatre pack marked separately from surgeon additions.	*Requests arrive from bookings without anybody raising them. Each case shows the standard theatre pack and, marked separately, what the surgeon added for this patient. That distinction matters: the additions are the ones that are unusual, the ones you may not stock, and the ones worth reading first.*
4 · 2:00–2:45 Sub-stores and reports	Sub-Stores view showing satellite stock; then Supply Reports with consumption trends; then Bulk Import loading a supplier delivery from a spreadsheet.	*Sub-Stores tracks what has gone out to satellite stores so a theatre cupboard is not invisible stock. Supply Reports shows consumption over time, which is the argument you need when asking for a budget. And Bulk Import loads a delivery from a spreadsheet rather than one line at a time.*
5 · 2:45–3:20 Codes	A patient at the store window presenting a consumable pack code; the pack list appearing.	*Patients arrive with a consumable pack code from their surgeon. Type it in and their list appears. It saves them explaining and saves you searching.*
6 · 3:20–3:30 Close	Presenter.	*One thing to do tomorrow: record every issue as you hand it over, not at the end of the day.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

```
V24 · Theatre store and supply

SHOT 01
IMAGE:     A store with full shelves. A theatre cancelled for a missing item that is on a shelf in the next room, in a box nobody opened.
MOTION:    Slow push in from a wide establishing frame
NARRATION: That item is in this room. It has been for three weeks. The case was cancelled anyway, because the shelf knew and the system did not.

SHOT 02
IMAGE:     Inventory screen: items, quantities, reorder levels, low-stock highlighted. A stock movement being recorded as items are issued.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Inventory holds what you have, what the reorder level is, and highlights what is running low. The balance is not typed — it is calculated from movements. So record issues and receipts as they happen, and the number stays true by itself. Overwrite a balance by hand and you have hidden the very thing the screen exists to show.

SHOT 03
IMAGE:     Theatre Supply Unit: consumable requests from booked cases, grouped by case and date, with the standard theatre pack marked separately from surgeon additions.
MOTION:    Gentle handheld, almost still — observational, not restless
NARRATION: Requests arrive from bookings without anybody raising them. Each case shows the standard theatre pack and, marked separately, what the surgeon added for this patient. That distinction matters: the additions are the ones that are unusual, the ones you may not stock, and the ones worth reading first.

SHOT 04
IMAGE:     Sub-Stores view showing satellite stock; then Supply Reports with consumption trends; then Bulk Import loading a supplier delivery from a spreadsheet.
MOTION:    Gentle handheld, almost still — observational, not restless
NARRATION: Sub-Stores tracks what has gone out to satellite stores so a theatre cupboard is not invisible stock. Supply Reports shows consumption over time, which is the argument you need when asking for a budget. And Bulk Import loads a delivery from a spreadsheet rather than one line at a time.

SHOT 05
IMAGE:     A patient at the store window presenting a consumable pack code; the pack list appearing.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Patients arrive with a consumable pack code from their surgeon. Type it in and their list appears. It saves them explaining and saves you searching.

SHOT 06
IMAGE:     Presenter.
MOTION:    Slow pull back, settling as the line lands
NARRATION: One thing to do tomorrow: record every issue as you hand it over, not at the end of the day.
```

```
VIDEO 24 - THEATRE STORE AND SUPPLY (3 min 30 s)
SCENES: 1 (0:00-0:20) Full store shelves; a case cancelled for an item that was in the next room.
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2 (0:20-1:05) Inventory with reorder levels and low stock; a movement recorded on issue. 3 (1:05-2:00) Theatre Supply Unit: requests grouped by case, standard pack separated from surgeon additions. 4 (2:00-2:45) Sub-Stores; Supply Reports consumption trends; Bulk Import from a spreadsheet. 5 (2:45-3:20) Patient presenting a consumable pack code at the window. 6 (3:20-3:30) Presenter.
NARRATION: paste the narration column, in order.

V25 · Consumable packs

Consumable pack provider | Scrub care provider | 2 min 30 s

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:18 Hook	A phone buzzing on a bench in the store at 17:40. The screen reads: new consumables pre-pack request.	*Twenty to six. The store is still open, the shelves are still reachable, and a request has just arrived for a case at nine tomorrow morning. This is the moment the case is either going to run or not.*
2 · 0:18–1:10 The work list	Consumable Packs screen: cases for tomorrow, each with its item list, quantities and the standard pack marked.	*Your screen is tomorrow's cases and what each one needs. You are told as soon as a case is booked, which is often days ahead. Work the list the evening before — every pack made up tonight is a case that cannot be delayed for a pack in the morning.*
3 · 1:10–1:55 Packing	Items ticked as packed; one item unavailable, marked with a note; the pack marked ready; the case status updating on the readiness board.	*Tick as you pack. If something is missing, mark it and write what you have put in instead — do not leave it blank and do not substitute silently. That note reaches the surgeon tonight. A surgeon told at six in the evening will adapt. A surgeon who discovers it at incision will not, and should not have to.*
4 · 1:55–2:20 Codes	A patient presenting a code; the pack found instantly; handed over and recorded.	*When a patient brings a code, type it in and her pack comes up. Record the hand-over so the theatre knows it has left you.*
5 · 2:20–2:30 Close	A row of labelled packs ready on a shelf. Presenter.	*One thing to do tomorrow: clear your list before you go home, not before you go to lunch.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

```
V25 · Consumable packs

SHOT 01
IMAGE:     A phone buzzing on a bench in the store at 17:40. The screen reads: new consumables pre-pack request.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Twenty to six. The store is still open, the shelves are still reachable, and a request has just arrived for a case at nine tomorrow morning. This is the moment the case is either going to run or not.

SHOT 02
IMAGE:     Consumable Packs screen: cases for tomorrow, each with its item list, quantities and the standard pack marked.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Your screen is tomorrow's cases and what each one needs. You are told as soon as a case is booked, which is often days ahead. Work the list the evening before — every pack made up tonight is a case that cannot be delayed for a pack in the morning.

SHOT 03
IMAGE:     Items ticked as packed; one item unavailable, marked with a note; the pack marked ready; the case status updating on the readiness board.
MOTION:    Hold steady on the screen, no camera move; highlight the tap target a beat before the finger arrives
NARRATION: Tick as you pack. If something is missing, mark it and write what you have put in instead — do not leave it blank and do not substitute silently. That note reaches the surgeon tonight. A surgeon told at six in the evening will adapt. A surgeon who discovers it at incision will not, and should not have to.

SHOT 04
IMAGE:     A patient presenting a code; the pack found instantly; handed over and recorded.
MOTION:    Slow push in
NARRATION: When a patient brings a code, type it in and her pack comes up. Record the hand-over so the theatre knows it has left you.

SHOT 05
IMAGE:     A row of labelled packs ready on a shelf. Presenter.
MOTION:    Slow pull back, settling as the line lands
NARRATION: One thing to do tomorrow: clear your list before you go home, not before you go to lunch.
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VIDEO 25 - CONSUMABLE PACKS (2 min 30 s)
SCENES: 1 (0:00-0:18) Phone buzzing in the store at 17:40: "new consumables pre-pack request". 2 (0:18-1:10) Consumable Packs screen with tomorrow's cases and item lists. 3 (1:10-1:55) Items ticked; one unavailable with a substitution note; pack marked ready; readiness board updating. 4 (1:55-2:20) Patient presenting a code; pack found and handed over. 5 (2:20-2:30) Labelled packs on a shelf.
NARRATION: paste the narration column, in order.
```

V26 · Procurement, vendors and finance

Procurement officer | Finance | 3 min

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:20 Hook	A meeting where somebody is asked how much a class of consumable costs per month and cannot answer.	*The question is reasonable and the answer is somewhere in six spreadsheets. By the time it is assembled, the decision has been taken without it.*
2 · 0:20–1:05 Inventory desk	Inventory Desk: low stock and reorder suggestions driven by actual consumption.	*Inventory Desk shows what is running down and what consumption says you will need, from movements that were recorded as the work happened rather than from an estimate at month end. Order against the trend rather than against the last complaint.*
3 · 1:05–1:50 Vendors	Vendor Accounts: suppliers, orders, deliveries, outstanding balances.	*Vendor Accounts holds suppliers, what was ordered, what arrived and what is owed. Keeping deliveries recorded here is what makes the next negotiation short, because the history is not in dispute.*
4 · 1:50–2:35 Finance and billing	Finance Desk; then Theatre Billing with a completed case and its itemised charges; then Cost Estimates for a planned case.	*Finance Desk and Theatre Billing turn what was actually used into what is actually charged, case by case, instead of a standard figure that is wrong in both directions. Cost Estimates does the same in advance, so a patient can be told what to expect before the day, which is the single most humane thing in this part of the system.*
5 · 2:35–3:00 Close	The same meeting, answered from a phone in ten seconds. Presenter.	*One thing to do this week: open Supply Reports and look at your three fastest-moving items.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

```
V26 · Procurement, vendors and finance

SHOT 01
IMAGE:     A meeting where somebody is asked how much a class of consumable costs per month and cannot answer.
MOTION:    Slow push in
NARRATION: The question is reasonable and the answer is somewhere in six spreadsheets. By the time it is assembled, the decision has been taken without it.

SHOT 02
IMAGE:     Inventory Desk: low stock and reorder suggestions driven by actual consumption.
MOTION:    Slow push in
NARRATION: Inventory Desk shows what is running down and what consumption says you will need, from movements that were recorded as the work happened rather than from an estimate at month end. Order against the trend rather than against the last complaint.

SHOT 03
IMAGE:     Vendor Accounts: suppliers, orders, deliveries, outstanding balances.
MOTION:    Slow push in
NARRATION: Vendor Accounts holds suppliers, what was ordered, what arrived and what is owed. Keeping deliveries recorded here is what makes the next negotiation short, because the history is not in dispute.

SHOT 04
IMAGE:     Finance Desk; then Theatre Billing with a completed case and its itemised charges; then Cost Estimates for a planned case.
MOTION:    Gentle handheld, almost still — observational, not restless
NARRATION: Finance Desk and Theatre Billing turn what was actually used into what is actually charged, case by case, instead of a standard figure that is wrong in both directions. Cost Estimates does the same in advance, so a patient can be told what to expect before the day, which is the single most humane thing in this part of the system.

SHOT 05
IMAGE:     The same meeting, answered from a phone in ten seconds. Presenter.
MOTION:    Slow pull back, settling as the line lands
NARRATION: One thing to do this week: open Supply Reports and look at your three fastest-moving items.
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VIDEO 26 - PROCUREMENT, VENDORS AND FINANCE (3 min)
SCENES: 1 (0:00-0:20) A meeting where a monthly cost question cannot be answered. 2 (0:20-1:05) Inventory Desk with consumption-driven reorder suggestions. 3 (1:05-1:50) Vendor Accounts: suppliers, orders, deliveries, balances. 4 (1:50-2:35) Finance Desk; Theatre Billing itemised for a completed case; Cost Estimates for a planned case. 5 (2:35-3:00) The same meeting, answered from a phone.
NARRATION: paste the narration column, in order.
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V27 · Porters

Porter | 2 min 30 s

PRODUCTION NOTE

Film entirely on a phone, walking, one-handed, in corridors. Nothing in this video should require sitting down or reading closely. Keep sentences very short.

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:18 Hook	A porter halfway down a corridor, turning back because he has been sent to the wrong ward.	*Wrong ward. Second time today. Both times somebody told him in a corridor, and both times they were remembering, not reading.*
2 · 0:18–1:05 Your jobs	Phone alerting; a transfer request showing patient name, ward, bed, destination theatre and how urgent it is. Accepted with one tap.	*Your phone tells you instead. The patient, the ward, the bed, which theatre, and how urgent. Tap to accept it so the theatre knows it is being done and stops asking. That one tap saves the nurses ringing round to find out whether anybody is coming.*
3 · 1:05–1:50 On the way	Patient Transfers screen: collected, then delivered, each tapped as it happens with the time recorded.	*Two more taps. Collected when you have the patient. Delivered when you hand over. That is the whole job in the app. It builds the honest record of how long transfers take, which is what gets more porters allocated when the list is heavy — nobody can argue with times that were recorded as they happened.*
4 · 1:50–2:20 Announcements	Corridor speaker announcing a patient by name and ward; the porter already walking.	*You will also hear it out loud. The radio announces the patient and the ward, so you often know before you look at the phone.*
5 · 2:20–2:30 Close	Delivering a patient calmly; presenter.	*One thing to do tomorrow: accept the job on your phone before you start walking.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

```
V27 · Porters

SHOT 01
IMAGE:     A porter halfway down a corridor, turning back because he has been sent to the wrong ward.
MOTION:    Slow dolly, travelling with the subject at walking pace
NARRATION: Wrong ward. Second time today. Both times somebody told him in a corridor, and both times they were remembering, not reading.

SHOT 02
IMAGE:     Phone alerting; a transfer request showing patient name, ward, bed, destination theatre and how urgent it is. Accepted with one tap.
MOTION:    Gentle handheld, almost still — observational, not restless
NARRATION: Your phone tells you instead. The patient, the ward, the bed, which theatre, and how urgent. Tap to accept it so the theatre knows it is being done and stops asking. That one tap saves the nurses ringing round to find out whether anybody is coming.

SHOT 03
IMAGE:     Patient Transfers screen: collected, then delivered, each tapped as it happens with the time recorded.
MOTION:    Hold steady on the screen, no camera move; highlight the tap target a beat before the finger arrives
NARRATION: Two more taps. Collected when you have the patient. Delivered when you hand over. That is the whole job in the app. It builds the honest record of how long transfers take, which is what gets more porters allocated when the list is heavy — nobody can argue with times that were recorded as they happened.

SHOT 04
IMAGE:     Corridor speaker announcing a patient by name and ward; the porter already walking.
MOTION:    Slow dolly, travelling with the subject at walking pace
NARRATION: You will also hear it out loud. The radio announces the patient and the ward, so you often know before you look at the phone.

SHOT 05
IMAGE:     Delivering a patient calmly; presenter.
MOTION:    Slow pull back, settling as the line lands
NARRATION: One thing to do tomorrow: accept the job on your phone before you start walking.
```

```
VIDEO 27 - PORTERS (2 min 30 s)
STYLE: shot entirely on a phone, walking, one-handed, in corridors. Very short sentences.
SCENES: 1 (0:00-0:18) Porter turning back, sent to the wrong ward. 2 (0:18-1:05) Phone alert with patient, ward, bed, destination theatre, urgency; accepted with one tap. 3 (1:05-1:50) Patient Transfers: collected, then delivered, times recorded. 4 (1:50-2:20) Corridor speaker announcing patient and ward; porter already walking. 5 (2:20-2:30) Calm delivery.
NARRATION: paste the narration column, in order.
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Part 7 — Engineering and utilities

Power, oxygen, water, equipment, laundry and meals. None of these departments see a patient, and every one of them can stop the list within a minute.

V28 · Biomedical engineering

Biomedical engineer | 3 min

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:20 Hook	An engineer stopped in a corridor by three people in turn, each describing a different fault from memory.	*Three faults, three corridors, no theatre numbers and no serial numbers. By the time he reaches the workshop he has one of them clearly and two of them approximately.*
2 · 0:20–1:10 Fault alerts	Fault Alerts list: each fault with theatre, equipment, who reported it, the time and a photograph. One opened, accepted, and marked in progress.	*Fault Alerts is that corridor, written down. Each one carries the theatre, the equipment, who reported it, when, and usually a photograph. Accept it so the reporter can see somebody has it — that single act stops the same fault being reported four more times by four more people.*
3 · 1:10–2:00 Equipment register	Equipment screen: devices with status, location, service history. A device set to out of service; theatre readiness reflecting it immediately.	*The Equipment register holds every device, where it is, whether it works and what has been done to it. When you take something out of service, set it here. The theatre readiness board changes at once, so the coordinator plans around it tonight instead of discovering it at eight tomorrow. Setting it back is just as important — a machine that is fixed but still marked broken is a machine nobody uses.*
4 · 2:00–2:40 Checkout and history	Equipment Checkout showing where a shared device is; then a service history with dates and interventions.	*Checkout tells you which room a shared device is in without opening six doors. And the service history is the argument for replacement: a machine with nine interventions in a year is a case you can make on paper rather than by insisting.*
5 · 2:40–3:00 Close	Fault closed; the reporter's screen showing it resolved. Presenter.	*One thing to do tomorrow: accept every new fault within the hour, even if you cannot start it yet.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

```
V28 · Biomedical engineering

SHOT 01
IMAGE:     An engineer stopped in a corridor by three people in turn, each describing a different fault from memory.
MOTION:    Slow dolly, travelling with the subject at walking pace
NARRATION: Three faults, three corridors, no theatre numbers and no serial numbers. By the time he reaches the workshop he has one of them clearly and two of them approximately.

SHOT 02
IMAGE:     Fault Alerts list: each fault with theatre, equipment, who reported it, the time and a photograph. One opened, accepted, and marked in progress.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Fault Alerts is that corridor, written down. Each one carries the theatre, the equipment, who reported it, when, and usually a photograph. Accept it so the reporter can see somebody has it — that single act stops the same fault being reported four more times by four more people.

SHOT 03
IMAGE:     Equipment screen: devices with status, location, service history. A device set to out of service; theatre readiness reflecting it immediately.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: The Equipment register holds every device, where it is, whether it works and what has been done to it. When you take something out of service, set it here. The theatre readiness board changes at once, so the coordinator plans around it tonight instead of discovering it at eight tomorrow. Setting it back is just as important — a machine that is fixed but still marked broken is a machine nobody uses.

SHOT 04
IMAGE:     Equipment Checkout showing where a shared device is; then a service history with dates and interventions.
MOTION:    Slow push in
NARRATION: Checkout tells you which room a shared device is in without opening six doors. And the service history is the argument for replacement: a machine with nine interventions in a year is a case you can make on paper rather than by insisting.

SHOT 05
IMAGE:     Fault closed; the reporter's screen showing it resolved. Presenter.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: One thing to do tomorrow: accept every new fault within the hour, even if you cannot start it yet.
```

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VIDEO 28 - BIOMEDICAL ENGINEERING (3 min)
SCENES: 1 (0:00-0:20) Engineer stopped by three people describing faults from memory. 2 (0:20-1:10) Fault Alerts with theatre, equipment, reporter, time, photograph; one accepted and set in progress. 3 (1:10-2:00) Equipment register; a device set out of service; readiness board updating. 4 (2:00-2:40) Equipment Checkout location; service history. 5 (2:40-3:00) Fault closed; reporter sees it resolved.
NARRATION: paste the narration column, in order.
```

V29 · Power house

Power plant operator | Works supervisor | 2 min 30 s

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:20 Hook	A theatre lit only by an overhead lamp, mid-case, for four seconds. Then full light returns.	*Four seconds. In a laparotomy, four seconds of darkness is a very long time. Whether it happens again this week is decided in a room most theatre staff have never visited.*
2 · 0:20–1:05 Status	Power Status: mains, generators, fuel levels, load, and which theatres are covered. Updated by the operator.	*Power Status is what the theatre suite sees about you. Mains, generators, fuel, load. Update it as things change. When theatre staff can see the fuel level themselves, they stop telephoning to ask, and they plan a long case around it instead of starting and hoping.*
3 · 1:05–1:50 Maintenance and readiness	Power Maintenance: scheduled servicing and test runs logged. Then Power Readiness showing green for the day's list.	*Log test runs and servicing in Power Maintenance as you do them. Power Readiness then answers, for today's list, whether the suite is properly backed. A planned test run recorded here means the theatre knows about it in advance. The same test run unannounced is an incident.*
4 · 1:50–2:20 Raising a problem	Fuel below threshold; an alert raised; the theatre manager notified.	*If fuel is low or a generator is unreliable, raise it here rather than reporting it upward verbally. It reaches the theatre manager with a time against it, and a shortage that is on record at ten in the morning gets solved. The same shortage mentioned in passing does not.*
5 · 2:20–2:30 Close	Steady lights; presenter.	*One thing to do tomorrow: update the fuel level at the start of every shift.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

```
V29 · Power house

SHOT 01
IMAGE:     A theatre lit only by an overhead lamp, mid-case, for four seconds. Then full light returns.
MOTION:    Slow push in from a wide establishing frame
NARRATION: Four seconds. In a laparotomy, four seconds of darkness is a very long time. Whether it happens again this week is decided in a room most theatre staff have never visited.

SHOT 02
IMAGE:     Power Status: mains, generators, fuel levels, load, and which theatres are covered. Updated by the operator.
MOTION:    Gentle handheld, almost still — observational, not restless
NARRATION: Power Status is what the theatre suite sees about you. Mains, generators, fuel, load. Update it as things change. When theatre staff can see the fuel level themselves, they stop telephoning to ask, and they plan a long case around it instead of starting and hoping.

SHOT 03
IMAGE:     Power Maintenance: scheduled servicing and test runs logged. Then Power Readiness showing green for the day's list.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Log test runs and servicing in Power Maintenance as you do them. Power Readiness then answers, for today's list, whether the suite is properly backed. A planned test run recorded here means the theatre knows about it in advance. The same test run unannounced is an incident.

SHOT 04
IMAGE:     Fuel below threshold; an alert raised; the theatre manager notified.
MOTION:    Gentle handheld, almost still — observational, not restless
NARRATION: If fuel is low or a generator is unreliable, raise it here rather than reporting it upward verbally. It reaches the theatre manager with a time against it, and a shortage that is on record at ten in the morning gets solved. The same shortage mentioned in passing does not.

SHOT 05
IMAGE:     Steady lights; presenter.
MOTION:    Slow pull back, settling as the line lands
NARRATION: One thing to do tomorrow: update the fuel level at the start of every shift.
```

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VIDEO 29 - POWER HOUSE (2 min 30 s)
SCENES: 1 (0:00-0:20) Theatre briefly dark mid-case, then full light. 2 (0:20-1:05) Power Status: mains, generators, fuel, load, theatres covered. 3 (1:05-1:50) Power Maintenance log; Power Readiness green for today. 4 (1:50-2:20) Low fuel alert raised; theatre manager notified. 5 (2:20-2:30) Steady lights.
NARRATION: paste the narration column, in order.
```

V30 · Oxygen control

Oxygen unit supervisor | 2 min 30 s

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:18 Hook	A cylinder gauge in the red, discovered by a technician at the start of a list.	*Discovered at ten to eight, by the person who needed it. There were four hours yesterday in which this was a small logistical problem.*
2 · 0:18–1:10 Stock and supply	Oxygen Control: cylinders by location and state, manifold pressure, consumption trend.	*Oxygen Control holds what you have, where it is and how fast it is going. Full, in use, empty, awaiting collection. Update as cylinders move. The consumption trend is the part worth watching — it tells you about a shortage days before the gauge does.*
3 · 1:10–1:55 Theatre visibility	The theatre readiness board showing an oxygen state; a theatre marked amber because supply is low.	*What you record appears on theatre readiness. That means a low supply is a decision the theatre makes in advance rather than a discovery it makes at the door, and it means nobody has to walk down to ask you.*
4 · 1:55–2:20 Escalation	Alert raised for delayed delivery; theatre manager and administration notified.	*If a delivery is late, raise it here. It goes to the theatre manager and to administration together, with a timestamp, so the escalation is already documented if it has to go further.*
5 · 2:20–2:30 Close	Full cylinders in place before the list; presenter.	*One thing to do tomorrow: record every cylinder change as you make it.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

```
V30 · Oxygen control

SHOT 01
IMAGE:     A cylinder gauge in the red, discovered by a technician at the start of a list.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Discovered at ten to eight, by the person who needed it. There were four hours
yesterday in which this was a small logistical problem.

SHOT 02
IMAGE:     Oxygen Control: cylinders by location and state, manifold pressure, consumption
trend.
MOTION:    Slow push in
NARRATION: Oxygen Control holds what you have, where it is and how fast it is going. Full, in
use, empty, awaiting collection. Update as cylinders move. The consumption trend is the part
worth watching — it tells you about a shortage days before the gauge does.

SHOT 03
IMAGE:     The theatre readiness board showing an oxygen state; a theatre marked amber
because supply is low.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: What you record appears on theatre readiness. That means a low supply is a
decision the theatre makes in advance rather than a discovery it makes at the door, and it
means nobody has to walk down to ask you.

SHOT 04
IMAGE:     Alert raised for delayed delivery; theatre manager and administration notified.
MOTION:    Gentle handheld, almost still — observational, not restless
NARRATION: If a delivery is late, raise it here. It goes to the theatre manager and to
administration together, with a timestamp, so the escalation is already documented if it has
to go further.

SHOT 05
IMAGE:     Full cylinders in place before the list; presenter.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: One thing to do tomorrow: record every cylinder change as you make it.
```

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VIDEO 30 - OXYGEN CONTROL (2 min 30 s)
SCENES: 1 (0:00-0:18) Cylinder gauge in the red found at the start of a list. 2 (0:18-1:10)
Oxygen Control: cylinders by location and state, manifold pressure, consumption trend. 3 (1:10-
1:55) Readiness board showing an amber oxygen state. 4 (1:55-2:20) Late-delivery alert raised. 5
(2:20-2:30) Full cylinders in place.
NARRATION: paste the narration column, in order.
```

V31 · Plumbing and water

Plumber | Plumbing / water supervisor | 2 min

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:18 Hook	A scrub sink running to a trickle while a surgeon waits to scrub.	*You cannot start a list without water at the scrub sink. This is the least glamorous screen in the whole application and one of the few that can stop everything.*
2 · 0:18–1:05 Status	Plumbing & Water: tank levels, pump status, supply to each theatre area, faults.	*Record tank levels, pump status and supply to each area. When it is on this screen the theatre suite can see it, which is the whole point — a scrub room that will be dry at nine tomorrow is something the coordinator can work around tonight.*
3 · 1:05–1:45 Faults	A leak raised with a photograph and location; assigned; then closed.	*Raise faults with a photograph and the exact location. Close them when they are done, because an open job that is actually finished makes every other number on the screen untrustworthy.*
4 · 1:45–2:00 Close	Water running properly; a surgeon scrubbing. Presenter.	*One thing to do tomorrow: record tank levels at the same time every morning.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

```
V31 · Plumbing and water

SHOT 01
IMAGE:     A scrub sink running to a trickle while a surgeon waits to scrub.
MOTION:    Slow push in
NARRATION: You cannot start a list without water at the scrub sink. This is the least
glamorous screen in the whole application and one of the few that can stop everything.

SHOT 02
IMAGE:     Plumbing & Water: tank levels, pump status, supply to each theatre area, faults.
MOTION:    Gentle handheld, almost still — observational, not restless
NARRATION: Record tank levels, pump status and supply to each area. When it is on this screen
the theatre suite can see it, which is the whole point — a scrub room that will be dry at
nine tomorrow is something the coordinator can work around tonight.

SHOT 03
IMAGE:     A leak raised with a photograph and location; assigned; then closed.
MOTION:    Slow push in
NARRATION: Raise faults with a photograph and the exact location. Close them when they are
done, because an open job that is actually finished makes every other number on the screen
untrustworthy.

SHOT 04
IMAGE:     Water running properly; a surgeon scrubbing. Presenter.
MOTION:    Slow pull back, settling as the line lands
NARRATION: One thing to do tomorrow: record tank levels at the same time every morning.
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VIDEO 31 - PLUMBING AND WATER (2 min)
SCENES: 1 (0:00-0:18) Scrub sink running to a trickle while a surgeon waits. 2 (0:18-1:05) Plumbing & Water: tank levels, pumps, supply by area, faults. 3 (1:05-1:45) A leak raised with photograph and location, assigned, then closed. 4 (1:45-2:00) Water running; surgeon scrubbing.
NARRATION: paste the narration column, in order.

V32 · Laundry, cleaning and meals

Laundry staff | Cleaner | Cafeteria manager | 3 min

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:20 Hook	A theatre between cases. Everything is ready except that the room has not been turned over, and nobody has been told the previous case finished.	*The case finished eleven minutes ago. The next patient is in the holding area. Nobody has told the person whose job it is to turn the room over, so eleven minutes is going to become thirty.*
2 · 0:20–1:05 Cleaning turnaround	A cleaner's phone alerting that a theatre needs turning over; accepted; marked complete; the readiness board turning green.	*When a case finishes, the room shows as needing cleaning, and that reaches you rather than waiting to be noticed. Accept it, do it, mark it done. The readiness board turns green and the coordinator can send for the next patient. Those two taps are worth twenty minutes a day on a busy list.*
3 · 1:05–1:50 Laundry	Laundry screen: linen sent, received, in process; stock of gowns and drapes; a shortage flagged.	*Laundry records what has gone out, what has come back and what is in stock. Flag a shortage as soon as you see it. Gowns and drapes are the kind of thing that is fine, fine, fine, and then stops a list on a Thursday morning — and the warning is always available a day before, on this screen.*
4 · 1:50–2:40 Meals	Theatre Meals showing numbers expected by shift; then Order Meal used by a nurse on a phone.	*Theatre Meals tells you how many people are actually on duty, from the roster, rather than from a guess. Staff order from their phones during the list. It sounds small next to the rest of this system. It is not: a team that has eaten runs a safer afternoon list, and a long emergency case is where that shows.*
5 · 2:40–3:00 Close	Fast, calm turnover between cases. Presenter.	*One thing to do tomorrow: mark the room done the moment it is done, not when you next pass a screen.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

```
V32 · Laundry, cleaning and meals

SHOT 01
IMAGE:     A theatre between cases. Everything is ready except that the room has not been turned over, and nobody has been told the previous case finished.
MOTION:    Slow push in from a wide establishing frame
NARRATION: The case finished eleven minutes ago. The next patient is in the holding area. Nobody has told the person whose job it is to turn the room over, so eleven minutes is going to become thirty.

SHOT 02
IMAGE:     A cleaner's phone alerting that a theatre needs turning over; accepted; marked complete; the readiness board turning green.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: When a case finishes, the room shows as needing cleaning, and that reaches you rather than waiting to be noticed. Accept it, do it, mark it done. The readiness board turns green and the coordinator can send for the next patient. Those two taps are worth twenty minutes a day on a busy list.

SHOT 03
IMAGE:     Laundry screen: linen sent, received, in process; stock of gowns and drapes; a shortage flagged.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Laundry records what has gone out, what has come back and what is in stock. Flag a shortage as soon as you see it. Gowns and drapes are the kind of thing that is fine, fine, fine, and then stops a list on a Thursday morning — and the warning is always available a day before, on this screen.

SHOT 04
IMAGE:     Theatre Meals showing numbers expected by shift; then Order Meal used by a nurse on a phone.
MOTION:    Gentle handheld, almost still — observational, not restless
NARRATION: Theatre Meals tells you how many people are actually on duty, from the roster, rather than from a guess. Staff order from their phones during the list. It sounds small next to the rest of this system. It is not: a team that has eaten runs a safer afternoon list, and a long emergency case is where that shows.

SHOT 05
IMAGE:     Fast, calm turnover between cases. Presenter.
MOTION:    Slow pull back, settling as the line lands
NARRATION: One thing to do tomorrow: mark the room done the moment it is done, not when you next pass a screen.
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VIDEO 32 - LAUNDRY, CLEANING AND MEALS (3 min)
SCENES: 1 (0:00-0:20) Theatre between cases, not turned over, next patient waiting. 2 (0:20-1:05) Cleaner's phone alert; accepted; marked complete; readiness board turning green. 3 (1:05-1:50) Laundry: linen sent, received, in process; stock; shortage flagged. 4 (1:50-2:40) Theatre Meals numbers from the roster; a nurse ordering from a phone. 5 (2:40-3:00) Fast calm turnover.
NARRATION: paste the narration column, in order.
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Part 8 — Leadership and administration

Theatre managers, chairmen, medical directors and system administrators. These videos are about seeing the whole suite, and about the responsibilities that come with being able to.

V33 · The theatre manager's day

Theatre manager | Theatre chairman | 3 min 30 s

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:20 Hook	A manager's office at 07:30. Three people waiting at the door, each with a different problem.	*Half past seven, and three people are already at the door. By nine she will have heard about the fourth problem, which started at six and which she could have seen last night.*
2 · 0:20–1:10 The evening view	Theatre Readiness and Case Pack Readiness open side by side the previous evening. Two amber rows.	*The job starts the night before, with two screens. Theatre Readiness for the rooms, Case Pack Readiness for the cases. Two amber rows at six in the evening are two phone calls. The same two rows at eight in the morning are a delayed list and three people at your door.*
3 · 1:10–2:00 Allocation	Theatre Allocation and Surgical Unit Calendar: units against theatres across the week; a unit moved to a different room.	*Allocation puts units into rooms across the week, and the Surgical Unit Calendar is how the units see it. Change it here and everyone sees the change — there is no separate list to circulate and no version confusion, which is where most of the arguments used to come from.*
4 · 2:00–2:50 Live day	Theatre Operations: live view of all theatres, cases in progress, milestones, delays highlighted. Emergency Response showing an unassigned emergency.	*Through the day, Theatre Operations is the suite in real time: what is running, what has started late and by how much. Emergency Response shows any emergency without a theatre or a team — that screen is the one to keep open, because an emergency nobody has taken is the most expensive thing on your list.*
5 · 2:50–3:20 Staff	Staff Availability and Duty Flyers; a gap in tomorrow's cover highlighted.	*Staff Availability shows who is available and where the gaps are. Duty Flyers publishes the day's assignments so nobody arrives asking where they are working.*
6 · 3:20–3:30 Close	Quiet office at 07:30, nobody waiting. Presenter.	*One thing to do tomorrow: look at both readiness screens at six in the evening, every evening.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

```
V33 · The theatre manager's day

SHOT 01
IMAGE:     A manager's office at 07:30. Three people waiting at the door, each with a different problem.
MOTION:    Slow push in from a wide establishing frame
NARRATION: Half past seven, and three people are already at the door. By nine she will have heard about the fourth problem, which started at six and which she could have seen last night.

SHOT 02
IMAGE:     Theatre Readiness and Case Pack Readiness open side by side the previous evening. Two amber rows.
MOTION:    Gentle handheld, almost still — observational, not restless
NARRATION: The job starts the night before, with two screens. Theatre Readiness for the rooms, Case Pack Readiness for the cases. Two amber rows at six in the evening are two phone calls. The same two rows at eight in the morning are a delayed list and three people at your door.

SHOT 03
IMAGE:     Theatre Allocation and Surgical Unit Calendar: units against theatres across the week; a unit moved to a different room.
MOTION:    Gentle handheld, almost still — observational, not restless
NARRATION: Allocation puts units into rooms across the week, and the Surgical Unit Calendar is how the units see it. Change it here and everyone sees the change — there is no separate list to circulate and no version confusion, which is where most of the arguments used to come from.

SHOT 04
IMAGE:     Theatre Operations: live view of all theatres, cases in progress, milestones, delays highlighted. Emergency Response showing an unassigned emergency.
MOTION:    Gentle handheld, almost still — observational, not restless
NARRATION: Through the day, Theatre Operations is the suite in real time: what is running, what has started late and by how much. Emergency Response shows any emergency without a theatre or a team — that screen is the one to keep open, because an emergency nobody has taken is the most expensive thing on your list.

SHOT 05
IMAGE:     Staff Availability and Duty Flyers; a gap in tomorrow's cover highlighted.
MOTION:    Slow push in
NARRATION: Staff Availability shows who is available and where the gaps are. Duty Flyers publishes the day's assignments so nobody arrives asking where they are working.

SHOT 06
IMAGE:     Quiet office at 07:30, nobody waiting. Presenter.
MOTION:    Slow pull back, settling as the line lands
NARRATION: One thing to do tomorrow: look at both readiness screens at six in the evening, every evening.
```

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VIDEO 33 - THE THEATRE MANAGER'S DAY (3 min 30 s)
SCENES: 1 (0:00-0:20) Manager's office 07:30, three people waiting. 2 (0:20-1:10) Theatre
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Readiness and Case Pack Readiness side by side the previous evening, two amber rows. 3 (1:10-2:00) Theatre Allocation and Surgical Unit Calendar; a unit moved. 4 (2:00-2:50) Theatre Operations live view with delays; Emergency Response showing an unassigned emergency. 5 (2:50-3:20) Staff Availability and Duty Flyers with a cover gap. 6 (3:20-3:30) Quiet office, nobody waiting.
NARRATION: paste the narration column, in order.
```

V34 · Publishing a departmental roster

Rota holders | Department heads | 3 min 30 s

SAY THIS PLAINLY IN THE VIDEO

A roster that is entered but **not published** does nothing at all. Draft rows are invisible to booking, to coverage and to the boards. Publishing is the step that makes it real, and forgetting it is the single most consequential mistake in this module.

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:25 Hook	A completed roster on screen with twenty-two rows, all marked draft. Beside it, tomorrow's theatre list showing "not yet assigned" against every case.	*Twenty-two people, entered correctly, for tomorrow. And tomorrow's list says nobody is assigned. The work was done. The last button was not pressed, and until it is, none of this exists.*
2 · 0:25–1:15 Entering	Duty Roster on a phone. Department chosen. A staff member added by typing three letters into a search-to-select field. Shift chosen: Electives or Call/Emergencies. Assignment chosen — a surgical specialty for anaesthetists, a theatre for technicians.	*Open Duty Roster and pick your department. Add a person by typing part of their name — the list finds them. Choose the shift: Electives, or Call and Emergencies. Then the assignment, and this is the field that matters most. For anaesthetists it is the surgical specialty they are covering. For technicians it is the theatre. That single field is what lets a booked case find the right person, so an empty one is a case that will read as unassigned tomorrow.*
3 · 1:15–2:00 Mobile first	The same form on a small phone screen, all fields visible, nothing cut off. Rows being added quickly, one after another.	*Do it from your phone if that is easier. Every field fits on the smallest screen — nothing is hidden behind a scroll, because most rosters in this hospital are submitted from a phone, often standing up, often at night.*
4 · 2:00–2:50 Publish	The pending-changes count showing 22 drafts and 1 staged removal. Publish pressed. Version number increments; rows turn published; the coverage screen fills with names.	*Now the important part. Your changes sit as drafts, and the counter tells you how many are waiting. Press Publish. The version number goes up, a permanent snapshot is taken so you can see exactly what was published and by whom, and only now does the rest of the hospital see it. Watch the coverage screen fill up. And notice: cases already booked pick up their proper anaesthetist automatically. You do not go back through them.*
5 · 2:50–3:20 Changes and history	A staged removal; a new version published; the versions list showing previous publications.	*Later changes work the same way: amend, then publish again. Every version is kept, so if somebody asks what the roster said on a particular day, the answer is not a memory.*
6 · 3:20–3:30 Close	All-green coverage; presenter.	*One thing to do tomorrow: after you enter a roster, look for the pending count. If it is not zero, you have not finished.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

```
V34 · Publishing a departmental roster

SHOT 01
IMAGE:     A completed roster on screen with twenty-two rows, all marked draft. Beside it,
tomorrow's theatre list showing "not yet assigned" against every case.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Twenty-two people, entered correctly, for tomorrow. And tomorrow's list says
nobody is assigned. The work was done. The last button was not pressed, and until it is, none
of this exists.

SHOT 02
IMAGE:     Duty Roster on a phone. Department chosen. A staff member added by typing three
letters into a search-to-select field. Shift chosen: Electives or Call/Emergencies.
Assignment chosen — a surgical specialty for anaesthetists, a theatre for technicians.
MOTION:    Hold steady on the screen, no camera move; highlight the tap target a beat before
the finger arrives
NARRATION: Open Duty Roster and pick your department. Add a person by typing part of their
name — the list finds them. Choose the shift: Electives, or Call and Emergencies. Then the
assignment, and this is the field that matters most. For anaesthetists it is the surgical
specialty they are covering. For technicians it is the theatre. That single field is what
lets a booked case find the right person, so an empty one is a case that will read as
unassigned tomorrow.

SHOT 03
IMAGE:     The same form on a small phone screen, all fields visible, nothing cut off. Rows
being added quickly, one after another.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Do it from your phone if that is easier. Every field fits on the smallest screen —
nothing is hidden behind a scroll, because most rosters in this hospital are submitted from a
phone, often standing up, often at night.

SHOT 04
IMAGE:     The pending-changes count showing 22 drafts and 1 staged removal. Publish pressed.
Version number increments; rows turn published; the coverage screen fills with names.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Now the important part. Your changes sit as drafts, and the counter tells you how
many are waiting. Press Publish. The version number goes up, a permanent snapshot is taken so
you can see exactly what was published and by whom, and only now does the rest of the
hospital see it. Watch the coverage screen fill up. And notice: cases already booked pick up
their proper anaesthetist automatically. You do not go back through them.

SHOT 05
IMAGE:     A staged removal; a new version published; the versions list showing previous
publications.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Later changes work the same way: amend, then publish again. Every version is kept,
so if somebody asks what the roster said on a particular day, the answer is not a memory.

SHOT 06
IMAGE:     All-green coverage; presenter.
MOTION:    Slow pull back, settling as the line lands
```

> NARRATION: One thing to do tomorrow: after you enter a roster, look for the pending count. If it is not zero, you have not finished.

VIDEO 34 - PUBLISHING A DEPARTMENTAL ROSTER (3 min 30 s)
SCENES: 1 (0:00-0:25) Twenty-two draft rows beside a theatre list reading "not yet assigned" on every case. 2 (0:25-1:15) Duty Roster on a phone: department, staff added by type-to-search, shift Electives or Call/Emergencies, assignment field (specialty for anaesthetists, theatre for technicians). 3 (1:15-2:00) Same form on a very small phone screen, all fields visible; rows added quickly. 4 (2:00-2:50) Pending count 22 drafts and 1 staged removal; Publish pressed; version increments; coverage screen fills with names. 5 (2:50-3:20) A staged removal; a new version; version history list. 6 (3:20-3:30) All-green coverage.
NARRATION: paste the narration column, in order.

V35 · Reports and analytics

Chairman | CMAC | Medical director | 3 min

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:20 Hook	A management meeting. Two people disagreeing about how many cases were cancelled last month; both are confident.	*Both of them are certain, and they differ by nineteen cases. Neither is lying. They are both remembering, and memory is not a data source.*
2 · 0:20–1:10 Reports	Reports & Analytics: case volume by unit, on-time starts, turnover times, cancellations by reason, theatre utilisation.	*Reports is built from timestamps recorded as the work happened. Case volume by unit. How many lists started on time. How long turnovers actually take. Cancellations, grouped by reason — and that last one is the most useful screen in the building, because it turns a complaint into a list you can act on.*
3 · 1:10–2:00 Performance and people	Theatre Ops Performance; then Staff Effectiveness.	*Performance shows where the time goes across the suite: which stage of the day loses it and where the queue forms. Staff Effectiveness does the same for people, and it deserves one caution said out loud. Use it to find where support is needed, not to rank colleagues. The moment staff believe it is a league table, they will start recording things late, and then every number here becomes worthless — including the ones you actually need.*
4 · 2:00–2:40 Research and presenting	Research & Analytics with a filtered cohort exported; then Presentation mode showing full-screen charts.	*Research lets you build a cohort and export it for study. Presentation puts the same figures on a screen for a meeting, so you are arguing from the system rather than from a slide somebody typed last night.*
5 · 2:40–3:00 Close	The meeting from scene 1, now looking at one screen together. Presenter.	*One thing to do this month: open cancellations by reason and fix the top one. It is almost always the cheapest thing on the list.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

```
V35 · Reports and analytics

SHOT 01
IMAGE:     A management meeting. Two people disagreeing about how many cases were cancelled last month; both are confident.
MOTION:    Slow push in
NARRATION: Both of them are certain, and they differ by nineteen cases. Neither is lying. They are both remembering, and memory is not a data source.

SHOT 02
IMAGE:     Reports & Analytics: case volume by unit, on-time starts, turnover times, cancellations by reason, theatre utilisation.
MOTION:    Gentle handheld, almost still — observational, not restless
NARRATION: Reports is built from timestamps recorded as the work happened. Case volume by unit. How many lists started on time. How long turnovers actually take. Cancellations, grouped by reason — and that last one is the most useful screen in the building, because it turns a complaint into a list you can act on.

SHOT 03
IMAGE:     Theatre Ops Performance; then Staff Effectiveness.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Performance shows where the time goes across the suite: which stage of the day loses it and where the queue forms. Staff Effectiveness does the same for people, and it deserves one caution said out loud. Use it to find where support is needed, not to rank colleagues. The moment staff believe it is a league table, they will start recording things late, and then every number here becomes worthless — including the ones you actually need.

SHOT 04
IMAGE:     Research & Analytics with a filtered cohort exported; then Presentation mode showing full-screen charts.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Research lets you build a cohort and export it for study. Presentation puts the same figures on a screen for a meeting, so you are arguing from the system rather than from a slide somebody typed last night.

SHOT 05
IMAGE:     The meeting from scene 1, now looking at one screen together. Presenter.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: One thing to do this month: open cancellations by reason and fix the top one. It is almost always the cheapest thing on the list.
```

```
VIDEO 35 - REPORTS AND ANALYTICS (3 min)
SCENES: 1 (0:00-0:20) Management meeting, two people disagreeing on cancellation numbers. 2 (0:20-1:10) Reports & Analytics: volume by unit, on-time starts, turnover, cancellations by reason, utilisation. 3 (1:10-2:00) Theatre Ops Performance; Staff Effectiveness with the caution about ranking. 4 (2:00-2:40) Research & Analytics cohort export; Presentation mode. 5 (2:40-3:00) The meeting now looking at one screen together.
NARRATION: paste the narration column, in order.
```

V36 · Administering the system

Administrator | System administrator | 3 min 30 s

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:20 Hook	A new registrar on her first day, unable to see the booking screen, standing behind somebody else who is booking for her.	*Day one. She cannot see the booking screen, so somebody else is booking under their own name for her. Every case she does this week will have the wrong person's name on it, and that is an administration problem, not a clinical one.*
2 · 0:20–1:10 Users	User Management: pending approvals, a new user approved, a role assigned, phone number and department checked.	*User Management is where staff are approved and given a role. Approve promptly — a clinician who cannot get in will borrow somebody else's login, and from that moment your audit trail is fiction. Check the phone number while you are there. It is what Chase on WhatsApp uses, and a wrong number there wastes somebody's morning.*
3 · 1:10–1:55 Module access	Module Access: granting one extra screen to a named user with a reason recorded.	*Roles cover most people, but not everyone. Module Access grants one extra screen to one named person, with a reason. Use it narrowly. The temptation is to give somebody the administrator role because it is faster, and that hands them everything, including things they should never see.*
4 · 1:55–2:40 Announcements and radio	Announcements: composing one, choosing when it plays, scheduling it daily. Then the radio playing it at the scheduled time.	*Announcements are what the suite hears. Write it, choose when it plays, and it is spoken at that time — once, at the right time, on the right day. Keep them few. An announcement that plays too often is an announcement people learn to ignore, and one day it will be the emergency alert they ignore.*
5 · 2:40–3:15 Theatre music	Theatre Music: uploading files into a category; the library filling; the player in the corner of the screen switching to it.	*Theatre Music is yours too. Upload tracks into categories and they appear in the player in the corner of every screen. It ducks automatically whenever the radio speaks, so it never covers an announcement. One responsibility comes with it: music played in a theatre is a public performance, so use recordings the hospital is entitled to play. The app cannot check that for you.*
6 · 3:15–3:30 Close	The registrar from scene 1, booking under her own name. Presenter.	*One thing to do tomorrow: clear the pending user approvals before you do anything else.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

```
V36 · Administering the system

SHOT 01
IMAGE:     A new registrar on her first day, unable to see the booking screen, standing behind somebody else who is booking for her.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Day one. She cannot see the booking screen, so somebody else is booking under their own name for her. Every case she does this week will have the wrong person's name on it, and that is an administration problem, not a clinical one.

SHOT 02
IMAGE:     User Management: pending approvals, a new user approved, a role assigned, phone number and department checked.
MOTION:    Slow push in
NARRATION: User Management is where staff are approved and given a role. Approve promptly — a clinician who cannot get in will borrow somebody else's login, and from that moment your audit trail is fiction. Check the phone number while you are there. It is what Chase on WhatsApp uses, and a wrong number there wastes somebody's morning.

SHOT 03
IMAGE:     Module Access: granting one extra screen to a named user with a reason recorded.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Roles cover most people, but not everyone. Module Access grants one extra screen to one named person, with a reason. Use it narrowly. The temptation is to give somebody the administrator role because it is faster, and that hands them everything, including things they should never see.

SHOT 04
IMAGE:     Announcements: composing one, choosing when it plays, scheduling it daily. Then the radio playing it at the scheduled time.
MOTION:    Slow push in
NARRATION: Announcements are what the suite hears. Write it, choose when it plays, and it is spoken at that time — once, at the right time, on the right day. Keep them few. An announcement that plays too often is an announcement people learn to ignore, and one day it will be the emergency alert they ignore.

SHOT 05
IMAGE:     Theatre Music: uploading files into a category; the library filling; the player in the corner of the screen switching to it.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Theatre Music is yours too. Upload tracks into categories and they appear in the player in the corner of every screen. It ducks automatically whenever the radio speaks, so it never covers an announcement. One responsibility comes with it: music played in a theatre is a public performance, so use recordings the hospital is entitled to play. The app cannot check that for you.

SHOT 06
IMAGE:     The registrar from scene 1, booking under her own name. Presenter.
MOTION:    Slow pull back, settling as the line lands
NARRATION: One thing to do tomorrow: clear the pending user approvals before you do anything else.
```

VIDEO 36 - ADMINISTERING THE SYSTEM (3 min 30 s)
SCENES: 1 (0:00-0:20) New registrar unable to see the booking screen, watching someone book for her. 2 (0:20-1:10) User Management: pending approvals, approve, assign role, check phone number. 3 (1:10-1:55) Module Access granting one screen to one named user with a reason. 4 (1:55-2:40) Announcements composed and scheduled; radio playing it at the right time. 5 (2:40-3:15) Theatre Music upload; library filling; player switching source; ducking under an announcement. 6 (3:15-3:30) Registrar booking under her own name.
NARRATION: paste the narration column, in order.

V37 · Imprest and billing

Imprest holders | Administration | 2 min 30 s

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:18 Hook	A drawer of receipts being emptied onto a desk at the end of a quarter.	*Three months of receipts, one afternoon, and at least four of them will be missing. Retirement does not have to be an archaeology exercise.*
2 · 0:18–1:05 The register	Imprest Register: an expense entered with amount, purpose and a photographed receipt attached.	*Enter each expense when you make it, and photograph the receipt straight onto the entry. Thirty seconds at the time replaces an afternoon at the end, and the receipt cannot be lost because it is no longer paper.*
3 · 1:05–1:50 Position and retirement	Quarterly Position showing spend against allocation; then Retirements assembling a complete return.	*Quarterly Position shows what has been spent against what was allocated, at any moment rather than at the end. Retirements assembles the return from entries that already exist, with the receipts attached.*
4 · 1:50–2:20 Duties and reports	Imprest Duties assigning responsibility; Imprest Reports showing a trend across quarters.	*Duties records who is responsible for what, so a query goes to a person rather than to a department. Reports show the trend across quarters, which is the case you make when the allocation is set.*
5 · 2:20–2:30 Close	Empty drawer; presenter.	*One thing to do tomorrow: photograph the receipt before you leave the counter.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

```
V37 · Imprest and billing

SHOT 01
IMAGE:     A drawer of receipts being emptied onto a desk at the end of a quarter.
MOTION:    Slow push in
NARRATION: Three months of receipts, one afternoon, and at least four of them will be
missing. Retirement does not have to be an archaeology exercise.

SHOT 02
IMAGE:     Imprest Register: an expense entered with amount, purpose and a photographed
receipt attached.
MOTION:    Slow push in
NARRATION: Enter each expense when you make it, and photograph the receipt straight onto the
entry. Thirty seconds at the time replaces an afternoon at the end, and the receipt cannot be
lost because it is no longer paper.

SHOT 03
IMAGE:     Quarterly Position showing spend against allocation; then Retirements assembling a
complete return.
MOTION:    Slow push in
NARRATION: Quarterly Position shows what has been spent against what was allocated, at any
moment rather than at the end. Retirements assembles the return from entries that already
exist, with the receipts attached.

SHOT 04
IMAGE:     Imprest Duties assigning responsibility; Imprest Reports showing a trend across
quarters.
MOTION:    Slow push in
NARRATION: Duties records who is responsible for what, so a query goes to a person rather
than to a department. Reports show the trend across quarters, which is the case you make when
the allocation is set.

SHOT 05
IMAGE:     Empty drawer; presenter.
MOTION:    Slow pull back, settling as the line lands
NARRATION: One thing to do tomorrow: photograph the receipt before you leave the counter.
```

```
VIDEO 37 - IMPREST AND BILLING (2 min 30 s)
SCENES: 1 (0:00-0:18) A drawer of receipts emptied onto a desk at quarter end. 2 (0:18-1:05)
Imprest Register entry with amount, purpose and photographed receipt. 3 (1:05-1:50) Quarterly
Position; Retirements assembled from existing entries. 4 (1:50-2:20) Imprest Duties; Imprest
Reports trend. 5 (2:20-2:30) Empty drawer.
NARRATION: paste the narration column, in order.
```

V38 · The safety net

All staff | 3 min

THE BRIEF

The last video in the series, and the one that decides whether staff trust the system with bad news. Tone must be calm and protective throughout.

SCENE	ON SCREEN	NARRATION
1 · 0:00–0:25 Hook	A junior member of staff noticing something wrong, looking at a senior colleague, and saying nothing.	*She saw it. She is the most junior person in the room, and she has decided it is not worth what it would cost to say. Every serious incident in every hospital in the world has this moment in it somewhere.*
2 · 0:25–1:10 Alerts and faults	Alerts screen; a fault raised from a theatre; an emergency alert going out.	*Most things are ordinary and go through Alerts and Fault Alerts. Something is broken, something is missing, something is not safe to use. Raise it with a photograph, and it goes to the people who can act with the time and the place attached. Nothing about that is a complaint. It is how the building learns.*
3 · 1:10–2:00 Anonymous tips	Anonymous Tips: a report being written with no name attached; the submission confirming that no identity was recorded.	*And some things are not ordinary. If you have seen something you are not comfortable putting your name to — unsafe practice, pressure to cut a corner, something worse — Anonymous Tips exists for exactly that. No name is recorded. It reaches the people who must know. Use it when you need it, and use it honestly. It is read.*
4 · 2:00–2:35 Feedback and mortality	Feedback form being submitted; then Mortality Registry and Security Reports.	*Feedback is for the app itself and for how the theatre runs — the suggestions screen is read by the people who build this. The Mortality Registry and Security Reports are the formal records, kept properly so that a review argues about the case and not about what happened.*
5 · 2:35–3:00 Close	The junior member of staff from scene 1, speaking up. The room listening. Presenter, final title card.	*A system is only as safe as the worst thing people are willing to report in it. One thing to do tomorrow: if something today felt wrong and you said nothing, write it down before you go home. That is the whole point of all thirty-eight of these videos.*

PASTE INTO PRODUCE — NARRATED SEQUENCE

One block per shot, in order. Paste **IMAGE** into your image generator, then **MOTION** and **NARRATION** into that shot's two fields.

```
V38 · The safety net

SHOT 01
IMAGE:     A junior member of staff noticing something wrong, looking at a senior colleague, and saying nothing.
MOTION:    Slow push in
NARRATION: She saw it. She is the most junior person in the room, and she has decided it is not worth what it would cost to say. Every serious incident in every hospital in the world has this moment in it somewhere.

SHOT 02
IMAGE:     Alerts screen; a fault raised from a theatre; an emergency alert going out.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Most things are ordinary and go through Alerts and Fault Alerts. Something is broken, something is missing, something is not safe to use. Raise it with a photograph, and it goes to the people who can act with the time and the place attached. Nothing about that is a complaint. It is how the building learns.

SHOT 03
IMAGE:     Anonymous Tips: a report being written with no name attached; the submission confirming that no identity was recorded.
MOTION:    Slow push in
NARRATION: And some things are not ordinary. If you have seen something you are not comfortable putting your name to — unsafe practice, pressure to cut a corner, something worse — Anonymous Tips exists for exactly that. No name is recorded. It reaches the people who must know. Use it when you need it, and use it honestly. It is read.

SHOT 04
IMAGE:     Feedback form being submitted; then Mortality Registry and Security Reports.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: Feedback is for the app itself and for how the theatre runs — the suggestions screen is read by the people who build this. The Mortality Registry and Security Reports are the formal records, kept properly so that a review argues about the case and not about what happened.

SHOT 05
IMAGE:     The junior member of staff from scene 1, speaking up. The room listening. Presenter, final title card.
MOTION:    Hold steady, no camera move; slow zoom to 150% on the field being discussed
NARRATION: A system is only as safe as the worst thing people are willing to report in it. One thing to do tomorrow: if something today felt wrong and you said nothing, write it down before you go home. That is the whole point of all thirty-eight of these videos.
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VIDEO 38 - THE SAFETY NET (3 min)
STYLE: calm and protective. Never accusatory.
SCENES: 1 (0:00-0:25) A junior staff member noticing something wrong and saying nothing. 2 (0:25-1:10) Alerts and Fault Alerts; a fault raised with a photograph; an emergency alert going out. 3 (1:10-2:00) Anonymous Tips written with no name; confirmation that no identity was recorded. 4 (2:00-2:35) Feedback submitted; Mortality Registry; Security Reports. 5 (2:35-3:00) The junior staff member speaking up; the room listening; final title card.
NARRATION: paste the narration column, in order.
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Appendix — Production checklist

BEFORE YOU RENDER	CHECK
Consistency	Same presenter, same voice, same lower-third style in all thirty-eight.
Device	Phone for anything done standing up; laptop for desk work. Never a desktop screenshot for a phone task.
Patient data	Only the fictional names in these scripts. No real folder numbers on screen, ever.
Length	Nothing over four minutes. If it runs long, split it.
Opening	A real problem in the first eight seconds. No "in this video we will".
Ending	Exactly one action for the next working day.
Tone	Read V3 scene 4 and V35 scene 3 aloud. If either sounds like a warning, rewrite it.
Accuracy	Re-shoot any screen that has changed since filming. A training video showing a screen that no longer exists costs more trust than it builds.